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Human Medicines Division

Pharmacovigilance Risk Assessment Committee (PRAC)

Minutes of PRAC meeting on 06-09 July 2026

Chair: Ulla Wändel Liminga – Vice-Chair: Liana Martirosyan

Health and safety information

In accordance with the Agency's health and safety policy, delegates were briefed on health, safety and emergency information and procedures prior to the start of the meeting.

Disclaimers

Some of the information contained in the minutes is considered commercially confidential or sensitive and therefore not disclosed. With regard to intended therapeutic indications or procedure scope listed against products, it must be noted that these may not reflect the full wording proposed by applicants and may also change during the course of the review. Additional details on some of these procedures will be published in the [PRAC meeting highlights](#) once the procedures are finalised.

Of note, the minutes are a working document primarily designed for PRAC members and the work the Committee undertakes.

Note on access to documents

Some documents mentioned in the minutes cannot be released at present following a request for access to documents within the framework of Regulation (EC) No 1049/2001 as they are subject to ongoing procedures for which a final decision has not yet been adopted. They will become public when adopted or considered public according to the principles stated in the Agency policy on access to documents ([EMA/127362/2006, Rev. 1](#)).

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1. Introduction

1.1. Welcome and declarations of interest of members, alternates and experts

The Chairperson opened the 06-09 July 2026 meeting by welcoming all participants. The meeting was held in-person.

In accordance with the Agency's policy on handling of declarations of interests of scientific Committees' members and experts, based on the declarations of interest submitted by the Committee members, alternates and experts and on the topics in the agenda of the meeting, the Committee Secretariat announced the restricted involvement of some Committee members, alternates and experts for concerned agenda topics. Participants were asked to declare any changes, omissions or errors to their declared interests concerning the matters for discussion. No new or additional competing interests were declared. Restrictions applicable to this meeting are captured in the List of participants included in the minutes.

Discussions, deliberations and voting took place in full respect of the restricted involvement of Committee members and experts in line with the relevant provisions of the Rules of Procedure (EMA/PRAC/567515/2012 Rev.3). All decisions taken at this meeting were made in the presence of a quorum of members. All decisions, recommendations and advice were agreed by consensus, unless otherwise specified. The members of the EEA-EFTA states agreed with the recommendation of the PRAC, unless otherwise specified.

The EMA Secretariat announced the names of the Committee members who delegated their vote via proxy and the Committee members who received such proxy.

The Chair announced the start of the Irish presidency of the Council of the European Union (EU).

1.2. Agenda of the meeting on 06-09 July 2026

The agenda was adopted with some modifications upon request from the members of the Committee and the EMA secretariat as applicable.

1.3. Minutes of the previous meeting on 08-11 June 2026

The minutes were adopted with some amendments received during the consultation phase and will be published on the EMA website.

Post-meeting note: the PRAC minutes of the meeting held on 08-11 June 2026 were published on the EMA website on 03 September 2026 ([EMA/PRAC/186037/2026](#)).

2. EU referral procedures for safety reasons: urgent EU procedures

2.1. Newly triggered procedures

None

2.2. Ongoing procedures

None

2.3. Procedures for finalisation

None

3. EU referral procedures for safety reasons: other EU referral procedures

3.1. Newly triggered procedures

None

3.2. Ongoing Procedures

None

3.3. Procedures for finalisation

None

3.4. Re-examination procedures¹

None

3.5. Others

None

4. Signals assessment and prioritisation²

For further details, see also the adopted [PRAC recommendations on signals](#) under the corresponding month.

4.1. New signals detected from EU spontaneous reporting systems and/or other sources

See also Annex I [14.1](#).

4.1.1. Atogepant - AQUIPTA (CAP); Erenumab – AIMOVIG (CAP); eptinezumab - VYEPTI (CAP); fremanezumab – AJOVY (CAP); galcanezumab – EMGALITY (CAP); rimegepant - VYDURA (CAP)

Applicant: AbbVie Deutschland GmbH & Co. KG (Aquirta), Eli Lilly Nederland B.V.

¹ Re-examination of PRAC recommendation under Article 32 of Directive 2001/83/EC

² Each signal refers to a substance or therapeutic class. The route of marketing authorisation is indicated in brackets (CAP for Centrally Authorised Products; NAP for Nationally Authorised Products including products authorised via Mutual Recognition Procedure and Decentralised Procedure). Product names are listed for reference Centrally Authorised Products (CAP) only. PRAC recommendations will specify the products concerned in case of any regulatory action required

(Emgality), H. Lundbeck A/S (Vyepti), Novartis Europharm Limited (Aimovig), Pfizer Europe MA EEIG (Vydura), TEVA GmbH (Ajovy)

PRAC Rapporteur: Terhi Lehtinen

Scope: Signal of Raynaud's phenomenon

EPITT 20292 – New signal

Lead Member State: DK, FI, LT, NL

Background

For background information on substance(s) and indication(s) of centrally authorised product(s) identified as 'CAP', see [Human medicine European public assessment report \(EPAR\)](#) on the EMA website.

During routine signal detection activities, a signal of Raynaud's phenomenon was identified by the Norwegian Medicines Agency (NOMA), based on cases reporting of positive de- or rechallenge or a dose-response-relationship. Denmark, Finland, Lithuania and The Netherlands confirmed that the signal needed initial analysis and prioritisation by PRAC.

Discussion

Having considered the available evidence from case reports in EudraVigilance and the literature, including the new cases of Raynaud's phenomenon reported after the previous signal review, PRAC agreed that further evaluation on the signal of Raynaud's phenomenon is warranted.

PRAC appointed Terhi Lehtinen as Rapporteur for the signal.

Summary of recommendation(s)

- The MAHs of Aimovig, Ajovy, Emgality, Vyepti, Vydura and Aquipta should submit by 14 October 2026 a cumulative review of all cases of Raynaud's phenomenon associated with calcitonin gene-related peptide (CGRP) antagonists erenumab, fremanezumab, galcanezumab, eptinezumab, rimegepant or atogepant as suspect medicinal products, including a review of the published literature, data from spontaneous reports and clinical studies, including all cases in EudraVigilance database, with data lock point of 30 July 2026. The MAHs should also discuss a possible biological plausibility and mechanism of this association, as well as the need for any potential amendment to the PI and/or the RMP and make accordingly a proposal for the changes to the relevant sections within this discussion.
- PRAC will assess the cumulative review within a 90-day timetable.

4.1.2. Oseltamivir – TAMIFLU (CAP); zanamivir – DECTOVA (CAP)

Applicant(s): Roche Registration GmbH (Tamiflu); GlaxoSmithKline Trading Services Limited (Dectova)

PRAC Rapporteur: Terhi Lehtinen

Scope: Signal of new information on the safety in patients critically ill with influenza

EPITT 20299 – New signal

Lead Member State: SE, FI

Background

For background information on substance(s) and indication(s) of centrally authorised product(s) identified as 'CAP', see [Human medicine European public assessment report \(EPAR\)](#) on the EMA website.

The MAH of Tamiflu (oseltamivir) informed EMA about the interim data from the investigator initiated REMAP-CAP trial (Randomised, Embedded, Multi-factorial, Adaptive Platform trial for Community-Acquired Pneumonia - global Bayesian adaptive platform trial assessing treatments in intensive care unit (ICU) patients with community-acquired pneumonia, with the aim of identifying interventions that improve patient outcomes) that were presented at the Critical Care Reviews Meeting in Belfast, Ireland (10th June 2026). The interim results of this study indicated increased mortality related to oseltamivir in ICU patients. In addition, the final results of post-authorisation efficacy study (PAES) study 208165 (retrospective, observational, chart review study to evaluate the clinical effectiveness of treatment with zanamivir 10 mg/ml solution for infusion in a cohort of ICU patients with complicated influenza infection) were considered of relevance, as the study was conducted in a cohort of ICU patients with complicated influenza infection. Therefore, Finland and Sweden confirmed that the signal needed initial analysis and prioritisation by PRAC.

Discussion

Having considered the available evidence from the REMAP-CAP trial and the PAES 208165, PRAC agreed that further evaluation on the signal of new information on safety in patients critically ill with influenza is warranted.

PRAC appointed Terhi Lehtinen as Rapporteur for the signal.

Summary of recommendation(s)

- The MAHs for Tamiflu (oseltamivir) and Dectova (zanamivir) should submit to EMA, by 26 August 2026, an assessment of the data available to each of them concerning the safety and impact on mortality in patients that are critically ill with influenza. Based on these data for the respective product, the MAHs should present and discuss available data on potential mechanisms whereby the product could be harmful for severely ill patients (including preclinical data concerning off-target effects). The MAHs should discuss the need for any potential amendment to the PI and/or the RMP, including the need for any further risk minimisation measures and make accordingly a proposal for changes to the relevant sections within this discussion.
- A 60-day timetable was recommended for the assessment of this review leading to a further PRAC recommendation.

4.1.3. Sevoflurane (NAP)

Applicant(s): various

PRAC Rapporteur: Eamon O Murchu

Scope: Signal of acute encephalopathy in patients carrying the mitochondrial DNA variant m.11232T>C

EPITT 20285 – New signal

Lead Member State: IE

Background

Sevoflurane is an inhalational anaesthetic agent indicated for induction and maintenance of general anaesthesia in adult and paediatric patients undergoing surgery.

During routine signal detection activities, a signal of acute encephalopathy in patients carrying the mitochondrial DNA variant m.11232T>C was identified by Spanish Medicines Agency (AEMPS), based on 8 cases from the national databases and 12 cases retrieved from EudraVigilance. Ireland confirmed that the signal needed initial analysis and prioritisation by PRAC.

Discussion

Having considered the available evidence from the case reports in national database, EudraVigilance and the literature, PRAC agreed that further evaluation on the signal of new aspect of the known risk of aseptic meningitis is warranted.

PRAC appointed Eamon O Murchu as Rapporteur for the signal.

Summary of recommendation(s)

- The MAHs for sevoflurane (ABBVIE SA, BAXTER SA and PIRAMAL CRITICAL CARE B.V.) should submit to EMA by 23/09/2026 a cumulative review and critical analysis of all cases of encephalopathy with basal ganglia degeneration following sevoflurane exposure in individuals with mitochondrial genetic variants, including the m.11232T>C (MT-ND4) variant. The review should include, but not be limited to, cases identified using the following MedDRA terms: HLGT 'Encephalopathies', SMQ broad 'Congenital, familial and genetic disorders', SMQ narrow 'Non-infectious encephalitis', and the PTs 'Central nervous system necrosis' and 'Basal ganglion degeneration', as well as any additional relevant MedDRA terms considered appropriate to identify pertinent cases in which sevoflurane was reported as the suspect medicinal product. This analysis should include a review of the published literature, data from spontaneous reports, including all cases in EudraVigilance database, and reports from studies. Based on the outcome of this review, the MAHs should discuss the need for any amendment to the PI and/or RMP and provide proposals for any relevant updates. Due consideration should be given to the practicality and feasibility of any proposed risk-minimisation measures, including the identification of at-risk individuals, the availability and feasibility of appropriate genetic testing, and the clinical context in which such testing could be implemented. Consideration should also be given to the need for additional risk communication measures, including a Direct Healthcare Professional Communication (DHPC), where appropriate.
- A 60-day timetable was recommended for the assessment of this review leading to a further PRAC recommendation.

4.2. Signals follow-up and prioritisation

4.2.1. Desogestrel (NAP), etonogestrel (NAP)

Applicant(s): various

PRAC Rapporteur: Karin Bolin

Scope: Signal of meningioma

Background

For background information, see PRAC minutes December 2025³.

The MAH submitted the requested data on the signal of meningioma and the responses were assessed by the Rapporteur.

Discussion

Having considered the available evidence in EudraVigilance and literature, including the data submitted by the MAH, PRAC agreed that a causal association between desogestrel and etonogestrel and meningioma is considered possible. Therefore, PRAC concluded that the product information should be updated to add a contraindication for patients with meningioma or history of meningioma, a warning on the increased risk of meningioma and to add meningioma as an undesirable effect with a frequency 'not known'.

Summary of recommendation(s)

- The MAHs of all desogestrel and etonogestrel containing products should submit to EMA, within 60 days, a variation to amend the product information⁴. PRAC also agreed on a direct healthcare professional communication (DHPC) together with a communication plan to inform healthcare professionals about a small increased risk of meningioma associated with current, prolonged use (>1 year) of desogestrel- or etonogestrel-containing contraceptives medicines.

4.2.2. Levonorgestrel intrauterine device 13.5 mg (Jaydess/Flereee) (NAP)

Applicant(s): various

PRAC Rapporteur: Dennis Lex

Scope: Signal of increased risk of ectopic pregnancy

EPITT 20251 – Follow-up to March 2026

Background

For background information, see PRAC minutes March 2026.

The MAH of Jaydess (Bayer) replied to the request for information on the signal of increased risk of ectopic pregnancy and the responses were assessed by the Rapporteur.

Discussion

Having considered the MAH's responses, PRAC has agreed that there is sufficient evidence to establish a causal association between use of levonorgestrel intrauterine device 13.5 mg and the risk of ectopic pregnancy. Therefore, PRAC concluded that the product information should be updated to reflect data from post-authorisation observational studies indicating a higher incidence rate of ectopic pregnancy with Jaydess (13.5 mg), compared to other intrauterine systems and copper intrauterine devices.

Summary of recommendation(s)

³ Held on 24-27 November 2025.

⁴ Update of sections 4.3, 4.4 and 4.8. The package leaflet is updated accordingly.

- The MAH for Jaydess (Bayer) should submit to EMA, within 60 days, a variation to update the product information⁵. The guide for risk minimisation for HCPs should be also updated to be in line with the product information (while the key elements remain unchanged) and is to be agreed at national level.

4.2.3. Venlafaxine (NAP)

Applicant(s): various

PRAC Rapporteur: Karin Bolin

Scope: Signal of cardiotoxicity

EPITT 20230 – Follow-up to December 2025

Background

For background information, see PRAC minutes December 2025⁶.

The innovator MAH of venlafaxine (Viatris Limited) replied to the request for information on the signal of cardiotoxicity and the responses were assessed by the Rapporteur.

Discussion

Having considered the available evidence from EudraVigilance, from the literature, the DARWIN study reports⁷ on real-world evidence data, as well as data provided by the innovator MAH, PRAC has concluded that the current evidence is insufficient to establish a causal relationship between venlafaxine and heart failure and cardiomyopathy to further warrant an update to the product information and/or risk management plan at present.

Summary of recommendation(s)

- The innovator MAH of venlafaxine should present a new review of literature data on the risk for cardiotoxicity (including heart failure and cardiomyopathy) in the next PSUR.

4.3. Variation procedure(s) resulting from signal evaluation

None

5. Risk management plans (RMPs)

5.1. Medicines in the pre-authorisation phase

PRAC provided advice to CHMP on the proposed RMPs for a number of products (identified by active substance below) that are under evaluation for initial marketing authorisation. Information on the PRAC advice will be available in the European Public Assessment Reports (EPARs) to be published at the end of the evaluation procedure.

Please refer to the CHMP pages for upcoming information

(<http://www.ema.europa.eu/Committees>CHMP>Agendas, minutes and highlights>).

⁵ Update of sections 4.4 and 4.8. The package leaflet is updated accordingly.

⁶ Held on 24-26 November 2025.

⁷ [DARWIN EU® - Assessing the potential association between venlafaxine and heart failure in adults | HMA-EMA Catalogues of real-world data sources and studies](https://catalogues.ema.europa.eu/node/4878/administrative-details) (<https://catalogues.ema.europa.eu/node/4878/administrative-details>)

See also Annex I [15.1](#).

5.1.1. Denecimig - (CAP MAA) - EMEA/H/C/006344

Scope (pre D-180 phase): Prophylaxis of bleeding episodes in patients with haemophilia A

5.1.2. Gefurulimab - (CAP MAA) - EMEA/H/C/006558

Scope (pre D-180 phase): Treatment of adult patients with generalised myasthenia gravis (gMG)

5.1.3. Navepegritide - (CAP MAA) - EMEA/H/C/006627, Orphan

Scope (pre D-180 phase): Treatment of achondroplasia in children

5.1.4. Povorcitinib - (CAP MAA) - EMEA/H/C/006727

Scope (pre D-180 phase): Treatment of active moderate to severe hidradenitis suppurativa (acne inversa) in adults

5.1.5. Relacorilant - (CAP MAA) - EMEA/H/C/006731, Orphan

Scope (pre D-180 phase): Treatment of adult patients with platinum-resistant epithelial ovarian, fallopian tube, or primary peritoneal cancer

5.1.6. Zamtocabtagene autoleucel - (CAP MAA) - EMEA/H/C/005495, PRIME, Orphan

ATMP

Scope (pre D-120 phase): Treatment of adults with large B-cell lymphoma

5.2. Medicines in the post-authorisation phase – PRAC-led procedures

See also Annex [15.2](#).

5.2.1. Adalimumab – HUMIRA (CAP) – EMA/VR/0000342430

Applicant: Abbvie Deutschland GmbH & Co. KG

PRAC Rapporteur: Karin Bolin

Scope: Submission of an updated RMP version 18 in order to request an early closure of PAM MEA 075, associated with Humira adult ulcerative colitis registry, Study P11-282, resulting in the change of the study milestone dates in the risk management plan. This registry study is listed as an additional pharmacovigilance activity (category 3 study) in the RMP.

Background

For background information on substance(s) and indication(s) of centrally authorised product(s) identified as 'CAP', see [Human medicine European public assessment report \(EPAR\)](#) on the EMA website.

PRAC is evaluating a type II variation procedure for Humira, a centrally authorised medicine containing adalimumab, to update the RMP to reflect the update of the study milestone dates for study P11-282. PRAC is responsible for adopting an outcome based on the assessment report from the PRAC Rapporteur, to be further considered at the level of CHMP, responsible for adopting an opinion on this variation.

Summary of advice

- The RMP version 18 for Humira (adalimumab) in the context of the variation under evaluation by PRAC and CHMP is considered acceptable.
- PRAC agreed with the MAH's proposal to shorten the planned duration of follow-up and thus submitting the final report in Q4 2026.

5.3. Medicines in the post-authorisation phase – CHMP-led procedures

See also Annex [15.3](#).

5.3.1. Osimertinib – TAGRISSO (CAP) – EMA/VR/0000340013

Applicant: AstraZeneca AB

PRAC Rapporteur: Bianca Mulder

Scope: Update of section 4.4 of the SmPC in order to update safety data based on post-marketing experience and literature. The Package Leaflet is updated accordingly. The RMP version 18.1 has also been submitted.

Background

For background information on substance(s) and indication(s) of centrally authorised product(s) identified as 'CAP', see [Human medicine European public assessment report \(EPAR\)](#) on the EMA website.

CHMP is evaluating a variation for Tagrisso, a centrally authorised product containing osimertinib, to update the product information based on post-marketing experience and literature. PRAC is responsible for providing advice to CHMP on the necessary updates to the RMP to support this procedure.

Summary of advice

- The RMP version 18.1 for Tagrisso (osimertinib) in the context of the variation under evaluation by CHMP is considered acceptable.
- PRAC supported the addition of rhabdomyolysis as a new important potential risk to the list of safety concerns and the inclusion of a questionnaire for adverse events associated with rhabdomyolysis as routine pharmacovigilance activity in the RMP.

5.3.2. Pandemic influenza vaccine (H5N1) (live attenuated, nasal) – PANDEMIC INFLUENZA VACCINE H5N1 ASTRAZENECA (CAP) – EMA/VR/0000321324

Applicant: AstraZeneca AB

PRAC Rapporteur: Sonja Radowan

Scope: Extension of indication to remove the upper age limit from the indication for Pandemic influenza vaccine (H5N1) (live, nasal), based on efficacy and safety data previously submitted in the Marketing Authorisation Application (MAA). As a consequence, sections 4.1, 4.2, 4.4, 4.5, 4.6, 4.8, 5.1, and 5.3 of the SmPC are updated. The Annex II and the Package Leaflet are updated in accordance. Version 2.2 of the RMP has also been submitted. In addition, the Marketing authorisation holder (MAH) took the opportunity to introduce editorial changes throughout the PI and update the list of local representatives in the Package Leaflet.

Background

For background information on substance(s) and indication(s) of centrally authorised product(s) identified as 'CAP', see [Human medicine European public assessment report \(EPAR\)](#) on the EMA website.

CHMP is evaluating an extension of the therapeutic indication for Pandemic influenza vaccine H5N1 Astrazeneca to remove the upper age limit from the indication. PRAC is responsible for providing advice to CHMP on the necessary updates to the RMP to support this procedure.

Summary of advice

- The RMP for Pandemic influenza vaccine H5N1 Astrazeneca in the context of the variation procedure under evaluation by CHMP could be considered acceptable provided that an update to RMP version 2.3 is submitted.
- PRAC considered that 'safety profile during pandemic use' should be included as missing information in the RMP, as the safety concerns currently addressed in the PASS study protocols relate both to all-cause serious adverse events and the overall safety profile. In addition, PRAC agreed that 'medically significant wheezing in children below 24 months of age' and 'vaccination failure (lack of efficacy)' should be removed from the list of safety concerns in the RMP.

5.3.3. Vosoritide – VOXZOGO (CAP) – EMA/VR/0000343152

Applicant: Biomarin International Limited

PRAC Rapporteur: Zane Neikena

Scope: Update of section 4.8 of the SmPC in order to add 'femur fracture' to the list of adverse drug reactions (ADRs) with frequency 'common' based on post-marketing data. The Package Leaflet is updated in accordance. The RMP version 4.0 has also been submitted. In addition, the MAH took the opportunity to update the PI in accordance with the latest EMA excipients guideline.

Background

For background information on substance(s) and indication(s) of centrally authorised product(s) identified as 'CAP', see [Human medicine European public assessment report \(EPAR\)](#) on the EMA website.

CHMP is evaluating a variation for Voxzogo, a centrally authorised product containing vosoritide, to update the product information to add femur fracture as an adverse reaction based on post-marketing data. PRAC is responsible for providing advice to CHMP on the necessary updates to the RMP to support this procedure.

Summary of advice

- The RMP for Voxzogo (vosoritide) in the context of the variation procedure under evaluation by CHMP could be considered acceptable provided that an update to RMP version 4.0 is submitted.
- PRAC considered that 'femur fractures' should be removed from the list of safety concerns of the RMP while continuing to be monitored as an adverse event of special interest in the ongoing studies and to be followed up in the upcoming PSURs. In addition, PRAC supported the removal of 'use in pregnancy' as safety concern from the RMP, as well as the removal of the pregnancy follow-up form from the pharmacovigilance plan.

6. Periodic safety update reports (PSURs)

6.1. PSUR single assessment (PSUSA) procedures including centrally authorised products (CAPs) only

For background information on substance(s) and indication(s) of centrally authorised product(s) identified as 'CAP', see [Human medicine European public assessment report \(EPAR\)](#) on the EMA website

See also Annex [16.1](#).

6.1.1. Afibercept – AFIVEG (CAP); AFQLIR (CAP); AHZANTIVE (CAP); BAIAMA (CAP); EIYZEY (CAP); EYDENZELT (CAP); EYLEA (CAP); EYLUXVI (CAP); MYNZEPLI (CAP); OPUVIZ (CAP); PAVBLU (CAP); VGENFLI (CAP); YESAFILI (CAP) – EMA/PSUR/0000336116

Applicants: Biolitec Pharma Limited, Bayer AG, Advanz Pharma Limited, ZAKLADY FARMACEUTYCZNE POLPHARMA S.A., Amgen Technology (Ireland) Unlimited Company, Celltrion Healthcare Hungary Kft., Formycon AG, Samsung Bioepis NL B.V., Sandoz GmbH, STADA Arzneimittel AG, Biosimilar Collaborations Ireland Limited

PRAC Rapporteur: Zoubida Amimour

Scope: Evaluation of a PSUSA procedure (PSUSA/00010020/202511)

Background

Based on the assessment of the PSUR, PRAC reviewed the benefit-risk balance of centrally authorised medicines containing aflibercept (ophthalmological indications) and issued a recommendation on their marketing authorisation(s).

Summary of recommendation(s) and conclusions

- Based on the review of the data on safety and efficacy, the benefit-risk balance of above-mentioned products in the approved indication(s) remains unchanged.
- Nevertheless, the product information should be updated to include retinal vasculitis and retinal occlusive vasculitis as undesirable effects with a frequency rare and unknown,

respectively. Therefore, the current terms of the marketing authorisation(s) should be varied⁸.

- In the next PSUR, the MAHs marketing the 8 mg formulation should provide a cumulative review of all preferred terms related to intraocular inflammation, stratified by seriousness, severity and outcome, and compared with the corresponding data for the 2 mg.
- The MAHs should reconsider the current list of safety concerns and re-evaluate the key elements of HCP guide and patient guide for aflibercept and should address these topics in the next RMP update to be submitted within an upcoming regulatory procedure affecting the RMP.

The next PSUR should be submitted in accordance with the requirements set out in the list of Union reference dates (EURD list) provided for under Article 107c(7) of Directive 2001/83/EC.

6.1.2. Enfortumab vedotin – PADCEV (CAP) – EMA/PSUR/0000336134

Applicant: Astellas Pharma Europe B.V.

PRAC Rapporteur: Eva Jirsová

Scope: Evaluation of a PSUSA procedure (PSUSA/00010989/202512)

Background

Based on the assessment of the PSUR, PRAC reviewed the benefit-risk balance of Padcev, a centrally authorised medicine containing enfortumab vedotin and issued a recommendation on its marketing authorisation(s).

Summary of recommendation(s) and conclusions

- Based on the review of the data on safety and efficacy, the benefit-risk balance of Padcev (enfortumab vedotin) in the approved indication(s) remains unchanged.
- Nevertheless, the product information should be updated to add opportunistic infections such as fungal infection as an undesirable effect with a frequency 'not known' for monotherapy and in combination with pembrolizumab. In addition, the product information should be updated to add leukopenia and urinary tract infection as undesirable effect with frequency 'common' and 'very common', respectively, for monotherapy and in combination with pembrolizumab. Therefore, the current terms of the marketing authorisation(s) should be varied⁹.
- In the next PSUR, the MAH should present new cases of pyelonephritis, lymphopenia drug reactions with eosinophilia and systemic symptoms (DRESS) and acute generalized exanthematous pustulosis (AGEP). In addition, the MAH should provide cumulative reviews of cases of insulin resistance and insulin resistant diabetes, as well as of cardiotoxicity with focus on arrhythmias, cardiac failure, ischaemic heart disease and myocarditis. The reviews should include a review of the published literature, data from spontaneous reports, as well as a discussion on possible biological plausibility and

⁸ Update of SmPC section 4.8. The package leaflet is updated accordingly. The PRAC AR and PRAC recommendation are transmitted to CHMP for adoption of an opinion

⁹ Update of SmPC sections 4.4 and 4.8. The package leaflet is updated accordingly. The PRAC AR and PRAC recommendation are transmitted to CHMP for adoption of an opinion

mechanism of this association. The MAH should also discuss the need for any potential amendment to the product information and / or the RMP. Additionally, the MAH should classify opportunistic infections as an important identified risk.

The next PSUR should be submitted in accordance with the requirements set out in the list of Union reference dates (EURD list) provided for under Article 107c(7) of Directive 2001/83/EC.

6.1.3. Ibrutinib – IMBRUVICA (CAP) – EMA/PSUR/0000336107

Applicant: Janssen Cilag International

PRAC Rapporteur: Barbara Kovacic Bytyqi

Scope: Evaluation of a PSUSA procedure (PSUSA/00010301/202511)

Background

Based on the assessment of the PSUR, PRAC reviewed the benefit-risk balance of Imbruvica, a centrally authorised medicine containing ibrutinib and issued a recommendation on its marketing authorisation(s).

Summary of recommendation(s) and conclusions

- Based on the review of the data on safety and efficacy, the benefit-risk balance of Imbruvica (ibrutinib) in the approved indication(s) remains unchanged.
- Nevertheless, the product information should be updated to add progressive multifocal leukoencephalopathy (PML) and cough as undesirable effects with a frequency rare and very common, respectively. The section 4.4 is updated to include cross-reference in line with the QRD template. Therefore, the current terms of the marketing authorisation(s) should be varied¹⁰. The RMP version 24.2 was also approved.
- In the next PSUR, the MAH should continue to monitor PML as identified risk in the PSUR and to present only newly identified interval cases of PML and associated terms. In addition, the MAH should present a cumulative review regarding the risk of haemophagocytic lymphohistiocytosis, as well as regarding pancreatitis, including cases from clinical trial cases, post-marketing cases, literature, non-clinical data and other relevant information, as well as a discussion on possible biological plausibility and mechanism of this association.

The frequency of PSUR submission should be revised from two-yearly to three-yearly and the next PSUR should be submitted to EMA within 90 days of the data lock point. The EURD list provided for under Article 107c(7) of Directive 2001/83/EC is updated accordingly.

6.1.4. Mosunetuzumab – LUNSUMIO (CAP) – EMA/PSUR/0000336142

Applicant: Roche Registration GmbH

PRAC Rapporteur: Jenny-Maria Jönsson

Scope: Evaluation of a PSUSA procedure (PSUSA/00010999/202512)

¹⁰ Update of SmPC sections 4.4 and 4.8. The package leaflet is updated accordingly. The PRAC AR and PRAC recommendation are transmitted to CHMP for adoption of an opinion

Background

Based on the assessment of the PSUR, PRAC reviewed the benefit-risk balance of Lunsumio, a centrally authorised medicine containing mosunetuzumab and issued a recommendation on its marketing authorisation(s).

Summary of recommendation(s) and conclusions

- Based on the review of the data on safety and efficacy, the benefit-risk balance of Lunsumio (mosunetuzumab) in the approved indication(s) remains unchanged.
- Nevertheless, the product information should be updated to include a warning on opportunistic infections and to add viral infections, bacterial infections, fungal infections as undesirable effects with frequencies as applicable (all grades/grades 3-4). Therefore, the current terms of the marketing authorisation(s) should be varied¹¹.

The next PSUR should be submitted in accordance with the requirements set out in the list of Union reference dates (EURD list) provided for under Article 107c(7) of Directive 2001/83/EC.

6.1.5. Ritlecitinib – LITFULO (CAP) – EMA/PSUR/0000336091

Applicant: Pfizer Europe MA EEIG

PRAC Rapporteur: Adam Przybylkowski

Scope: Evaluation of a PSUSA procedure (PSUSA/00000133/202512)

Background

Based on the assessment of the PSUR, PRAC reviewed the benefit-risk balance of Litfulo, a centrally authorised medicine containing ritlecitinib and issued a recommendation on its marketing authorisation(s).

Summary of recommendation(s) and conclusions

- Based on the review of the data on safety and efficacy and taking into account the MAH position discussed in the context of the oral explanation held on 07 July 2026, the benefit-risk balance of Litfulo (ritlecitinib) in the approved indication(s) remains unchanged.
- Nevertheless, the product information should be updated to introduce a boxed warning recommending the use of ritlecitinib only if no suitable treatment alternatives are available in patients 65 years of age and older, with history of atherosclerotic cardiovascular disease or other cardiovascular risk factors, who are current or past long-time smoker and with other malignancy risk factors (e.g. current, or history of malignancy). In addition, the product information should be updated to amend the warnings on 'serious infections', 'malignancy (including non-melanoma skin cancer)', and 'major adverse cardiovascular events (MACE), deep venous thrombosis (DVT) and pulmonary embolism (PE)' and to add a warning regarding use in patients 65 years and older. Therefore, the current terms of the marketing authorisation(s) should be varied¹².

¹¹ Update of SmPC sections 4.4 and 4.8. The package leaflet is updated accordingly. The PRAC AR and PRAC recommendation are transmitted to CHMP for adoption of an opinion

¹² Update of SmPC section 4.4. The package leaflet is updated accordingly. Annex II-D was also updated. The PRAC AR and PRAC recommendation are transmitted to CHMP for adoption of an opinion

- PRAC also agreed on the distribution of a direct healthcare professional communication (DHPC) together with a communication plan, to inform healthcare professionals of the outcome of the review and associated risk minimisation measures for Litfulo (ritlecitinib).
- In addition, PRAC recommended updates of the key elements of the existing educational materials in line with the proposed updates of the product information, to update the existing PASS (B7981101) in place to add 'all-cause mortality' as a safety endpoint, and to update the existing drug utilisation study (B7981102) to add the risk factor of MACE and to evaluate the effectiveness of the newly recommended risk minimisation measures. The important potential risk of 'thromboembolic events including deep vein thrombosis, pulmonary embolism and arterial thrombosis' was also renamed to 'venous thromboembolism and arterial thrombosis'. The RMP version 2.2 was considered acceptable by PRAC.

The next PSUR should be submitted in accordance with the requirements set out in the list of Union reference dates (EURD list) provided for under Article 107c(7) of Directive 2001/83/EC.

6.1.6. **Roxadustat – EVRENZO (CAP) – EMA/PSUR/0000336126**

Applicant: Astellas Pharma Europe B.V.

PRAC Rapporteur: Jana Pecherova

Scope: Evaluation of a PSUSA procedure (PSUSA/00010955/202512)

Background

Based on the assessment of the PSUR, PRAC reviewed the benefit-risk balance of Evrenzo, a centrally authorised medicine containing roxadustat and issued a recommendation on its marketing authorisation(s).

Summary of recommendation(s) and conclusions

- Based on the review of the data on safety and efficacy, the benefit-risk balance of Evrenzo (roxadustat) in the approved indication(s) remains unchanged.
- The current terms of the marketing authorisation(s) should be maintained.
- In the next PSUR, the MAH should further monitor the safety topics vaso-occlusive crisis and myocardial infarction and closely monitor reports concerning a potential interaction between roxadustat and hydroxyurea. The MAH should closely monitor cases of hepatic events and should provide a detailed cumulative review of cases of restless legs syndrome reported in clinical trials and in post-marketing data along with a literature review, and provide a discussion on the potential pharmacological and physiological mechanisms, as well as on the need for an update of the product information.

The next PSUR should be submitted in accordance with the requirements set out in the list of Union reference dates (EURD list) provided for under Article 107c(7) of Directive 2001/83/EC.

6.1.7. **Sugemalimab – CEJEMLY (CAP) – EMA/PSUR/0000336144**

Applicant: Cstone Pharmaceuticals Ireland Limited

PRAC Rapporteur: Petar Mas

Scope: Evaluation of a PSUSA procedure (PSUSA/00011080/202512)

Background

Based on the assessment of the PSUR, PRAC reviewed the benefit-risk balance of Cejemly, a centrally authorised medicine containing sugemalimab and issued a recommendation on its marketing authorisation(s).

Summary of recommendation(s) and conclusions

- Based on the review of the data on safety and efficacy, the benefit-risk balance of Cejemly (sugemalimab) in the approved indication(s) remains unchanged.
- Nevertheless, the product information should be updated to add a warning regarding immune-related adverse reactions in patients with pre-existing autoimmune disease. Therefore, the current terms of the marketing authorisation(s) should be varied¹³.

The next PSUR should be submitted in accordance with the requirements set out in the list of Union reference dates (EURD list) provided for under Article 107c(7) of Directive 2001/83/EC.

6.1.8. Tabelecleucel – EBVALLO (CAP) – EMA/PSUR/0000336145

Applicant: Pierre Fabre Medicament

PRAC Rapporteur: Amelia Cupelli

Scope: Evaluation of a PSUSA procedure (PSUSA/00011028/202512)

Background

Based on the assessment of the PSUR, PRAC reviewed the benefit-risk balance of Ebvallo, a centrally authorised medicine containing tabelecleucel and issued a recommendation on its marketing authorisation(s).

Summary of recommendation(s) and conclusions

- Based on the review of the data on safety and efficacy, the benefit-risk balance of Ebvallo (tabelecleucel) in the approved indication(s) remains unchanged.
- Nevertheless, the product information should be updated to add cytokine release syndrome (CRS) as an undesirable effect with a frequency 'common'. Therefore, the current terms of the marketing authorisation(s) should be varied¹⁴.
- In the next PSUR, the MAH should continue close monitoring of immune effector cell-associated neurotoxicity syndrome (ICANS) as an important potential risk and provide a discussion of any newly reported case from any source and follow-up information of previously reported cases, if any. The discussion should include the differential diagnosis and any relevant clinical, radiological or biomarker data supporting the diagnostic and causality assessment.

¹³ Update of SmPC section 4.4. The package leaflet is updated accordingly. The PRAC AR and PRAC recommendation are transmitted to CHMP for adoption of an opinion

¹⁴ Update of SmPC section 4.8. The package leaflet is updated accordingly. The PRAC AR and PRAC recommendation are transmitted to CHMP for adoption of an opinion

- The MAH should also update the RMP to reflect the reclassification of cytokine release syndrome from an important potential risk to an important identified risk.

The next PSUR should be submitted in accordance with the requirements set out in the list of Union reference dates (EURD list) provided for under Article 107c(7) of Directive 2001/83/EC.

6.1.9. Tislelizumab – TEVIMBRA (CAP) – EMA/PSUR/0000336087

Applicant: Beone Medicines Ireland Limited

PRAC Rapporteur: Bianca Mulder

Scope: Evaluation of a PSUSA procedure (PSUSA/00000136/202512)

Background

Based on the assessment of the PSUR, PRAC reviewed the benefit-risk balance of Tevimbra, a centrally authorised medicine containing tislelizumab issued a recommendation on its marketing authorisation(s).

Summary of recommendation(s) and conclusions

- Based on the review of the data on safety and efficacy, the benefit-risk balance of Tevimbra (tislelizumab) in the approved indication(s) remains unchanged.
- Nevertheless, the product information should be updated to add a dose recommendation and warning regarding myocarditis-myositis-myasthenia gravis overlap syndrome, as well as to add myocarditis-myositis-myasthenia gravis overlap syndrome as an undesirable effect with a frequency 'uncommon'. Therefore, the current terms of the marketing authorisation(s) should be varied¹⁵.

The frequency of PSUR submission should be revised from yearly to three-yearly and the next PSUR should be submitted to EMA within 90 days of the data lock point. The EURD list provided for under Article 107c(7) of Directive 2001/83/EC is updated accordingly.

6.2. PSUR single assessment (PSUSA) procedures including centrally authorised products (CAPs) and nationally authorised products (NAPs)

See Annex [16.2](#).

6.3. PSUR single assessment (PSUSA) procedures including nationally authorised products (NAPs) only

See also Annex [16.3](#)

6.3.1. Caffeine / dimenhydrinate, dimenhydrinate – EMA/PSUR/0000336086

Applicants: various

PRAC Lead: Maria Martinez Gonzalez

¹⁵ Update of SmPC sections 4.2, 4.4 and 4.8. The package leaflet is updated accordingly. The PRAC AR and PRAC recommendation are transmitted to CHMP for adoption of an opinion

Scope: Evaluation of a PSUSA procedure (PSUSA/00001088/202512)

Background

Dimenhydrinate is an ethanolamine with antihistamine properties. It also has anticholinergic (antimuscarinic) properties that explain the antiemetic effect of the drug in cases of labyrinthine irritation (nausea, vertigo), as well as the central sedation. Caffeine has stimulating effects.

Based on the assessment of the PSUR(s), PRAC reviewed the benefit-risk balance of nationally authorised medicine(s) containing caffeine / dimenhydrinate, dimenhydrinate and issued a recommendation on their marketing authorisation(s).

Summary of recommendation(s) and conclusions

- Based on the review of the data on safety and efficacy, the benefit-risk balance of caffeine / dimenhydrinate, dimenhydrinate-containing medicinal products in the approved indication(s) remains unchanged.
- Nevertheless, the product information should be updated to add warnings regarding drug abuse, and severe cutaneous adverse reactions (SCARs) and Stevens–Johnson syndrome (SJS). The product information should also be updated to add Stevens–Johnson syndrome (SJS) and fixed drug eruption with frequencies ‘not known’. In addition, the product information should be updated to include information on potential for additive anticholinergic effects with anti-parkinsonian antimuscarinics and other anticholinergic medicinal products. Therefore, the current terms of the marketing authorisation(s) should be varied¹⁶.
- In the next PSUR, all MAHs should continue monitoring overdose and/or medication errors in paediatric patients. In addition, all MAHs should provide a cumulative review of cases of toxic epidermal necrolysis, as well as a cumulative review of European drug abuse cases.

The next PSUR should be submitted in accordance with the requirements set out in the list of Union reference dates (EURD list) provided for under Article 107c(7) of Directive 2001/83/EC.

6.3.2. Choline salicylate – EMA/PSUR/0000336077

Applicants: various

PRAC Lead: Adam Przybylkowski

Scope: Evaluation of a PSUSA procedure (PSUSA/00000732/202512)

Background

Choline salicylate, the choline salt of salicylic acid, is a topical NSAID that has anti-inflammatory and analgesic properties. It is indicated, for topical otic and oromucosal use, for the treatment of pain from infections or inflammations of the ear, as well as treatment of pain inflammation, minor superficial injuries, or ulcers in mouth and throat.

¹⁶ Update of SmPC sections 4.4, 4.5 and 4.8. The package leaflet is updated accordingly. The PRAC AR and PRAC recommendation are transmitted to CMDh for adoption of a position

Based on the assessment of the PSUR(s), PRAC reviewed the benefit-risk balance of nationally authorised medicine(s) containing choline salicylate and issued a recommendation on their marketing authorisation(s).

Summary of recommendation(s) and conclusions

- Based on the review of the data on safety and efficacy, the benefit-risk balance of choline salicylate-containing medicinal products in the approved indication(s) remains unchanged.
- Nevertheless, the product information should be updated to add a warning regarding the use in patients with a history of or a current peptic ulcer for choline salicylate oromucosal products, to add a warning on the risk of using aspirin or other products containing salicylates and to add a warning regarding the use in pregnancy for choline salicylate containing medicinal products. In case the product information already includes a similar or stricter advice on use in pregnancy, the similar or stricter advice remains valid and should remain. Therefore, the current terms of the marketing authorisation(s) should be varied¹⁷.
- In the next PSURs, all MAHs should continue to monitor the following safety issues: hypersensitivity reactions, Reye's syndrome in children, use during pregnancy and use during breastfeeding.

One single entry in the EURD list for monocomponent choline salicylate and fixed-dose combination cetalkonium/choline salicylate and changes PSUR frequency are proposed. The PSUR submission frequency for monocomponent choline salicylate products should be adjusted to a fixed-dose combination of cetalkonium/choline salicylate products, i.e., 8 years, with the next DLP on 03.08.2033.

6.3.3. Phenylephrine / tropicamide – EMA/PSUR/0000336103

Applicants: various

PRAC Lead: Karin Erneholm

Scope: Evaluation of a PSUSA procedure (PSUSA/00010430/202511)

Background

Phenylephrine / tropicamide is an ophthalmic combination product promoting mydriasis. Phenylephrine is a sympathomimetic agent with affinity for alpha-1-adrenergic receptors, and tropicamide is an anticholinergic agent acting as a non-selective antagonist of cholinergic muscarinic receptors. The combination of tropicamide (with phenylephrine is indicated for mydriasis for the examination of the optic fundus, diagnosis of errors of refraction, iritis, iridocyclitis, and uveitis (eye drops), or to obtain pre-operative mydriasis or for diagnostic purposes when monotherapy is known to be insufficient (ophthalmic inserts).

Based on the assessment of the PSUR(s), PRAC reviewed the benefit-risk balance of nationally authorised medicine(s) containing phenylephrine/tropicamide and issued a recommendation on their marketing authorisation(s).

Summary of recommendation(s) and conclusions

¹⁷ Update of SmPC sections 4.4 and 4.6. The package leaflet is updated accordingly. The PRAC AR and PRAC recommendation are transmitted to CMDh for adoption of a position

- Based on the review of the data on safety and efficacy, the benefit-risk balance of phenylephrine/tropicamide-containing medicinal products in the approved indication(s) remains unchanged.
- Nevertheless, the product information should be updated to add a warning regarding necrotising enterocolitis in premature infants and to add anaphylactic reaction and necrotising enterocolitis in premature infants as undesirable effects with a frequency 'not known'. Therefore, the current terms of the marketing authorisation(s) should be varied¹⁸.

The frequency of PSUR submission should be revised from five-yearly to seven-yearly and the next PSUR should be submitted to EMA within 90 days of the data lock point. The EURD list provided for under Article 107c(7) of Directive 2001/83/EC is updated accordingly.

6.4. Follow-up to PSUR/PSUSA procedures

See Annex [16.4](#).

6.5. Variation procedure(s) resulting from PSUSA evaluation

See Annex [16.5](#).

6.6. Expedited summary safety reviews¹⁹

None

7. Post-authorisation safety studies (PASS)

7.1. Protocols of PASS imposed in the marketing authorisation(s)²⁰

See Annex [17.1](#).

7.2. Protocols of PASS non-imposed in the marketing authorisation(s)²¹

See Annex [17.2](#).

7.3. Results of PASS imposed in the marketing authorisation(s)²²

7.3.1. Blinatumomab – BLINCYTO (CAP) – EMA/PASS/0000262863

Applicant: Amgen Europe B.V.

PRAC Rapporteur: Veronika Macurova

¹⁸ Update of SmPC sections 4.4 and 4.8. The package leaflet is updated accordingly. The PRAC AR and PRAC recommendation are transmitted to CMDh for adoption of a position

¹⁹ Submission of expedited summary safety reports for review in addition to the requirements for submission of PSUR(s) falling within the pandemic period and requirements set out in the list of Union reference dates (EURD list) provided for under Article 107c(7) of Directive 2001/83/EC

²⁰ In accordance with Article 107n of Directive 2001/83/EC

²¹ In accordance with Article 107m of Directive 2001/83/EC, supervised by PRAC in accordance with Article 61a (6) of Regulation (EC) No 726/2004

²² In accordance with Article 107p-q of Directive 2001/83/EC

Scope: PASS results [107q]: Final study report for an observational study of blinatumomab safety and effectiveness, utilisation, and treatment practices

Background

For background information on substance(s) and indication(s) of centrally authorised product(s) identified as 'CAP', see [Human medicine European public assessment report \(EPAR\)](#) on the EMA website.

In order to fulfil the obligation to submit the results of an imposed non-interventional PASS (EUPAS17848) in accordance with Article 107p of Directive 2001/83/EC, the MAH submitted on 25 March 2025 a PASS final study report to the EMA for blinatumomab. PRAC discussed the final study results in addition to the MAH's responses to the request for supplementary information (RSI).

Summary of recommendation(s) and conclusions

- Based on the review of the final report of the non-interventional PASS entitled 'An Observational Study of Blinatumomab Safety and Effectiveness, Utilisation, and Treatment Practices', PRAC considered that the benefit-risk balance of Blincyto (blinatumomab) remains unchanged. Nevertheless, PRAC concluded that the product information should be updated to reflect the results of the study and to remove the statements on limited efficacy data in late relapsing (LFR) patients. No changes are proposed for the package leaflet. PRAC also agreed that the following safety concerns to be removed from the RMP: opportunistic infections, use in patients after recent allogeneic haematopoietic stem cell transplantation (HSCT), recent or concomitant treatment with other anti-cancer therapies (including radiotherapy) and recent or concomitant treatment with other immunotherapy. Finally, PRAC recommended that the terms of the marketing authorisation(s) for Blincyto (blinatumomab) should be varied to remove the PASS from the 'conditions or restrictions with regard to the safe and effective use of the medicinal product'.

7.3.2. Sodium valproate (NAP) – EMA/PASS/0000287665

Applicants: various

PRAC Rapporteur: Liana Martirosyan

Scope: valproate PASS results [107q]: Final study results for Drug Utilisation Study extension of valproate and related substances, in Europe, using databases.

Background

Valproic acid and the related substances are antiepileptics indicated for the treatment of epilepsy, of bipolar disorders restricted to the treatment of manic episodes when lithium is contraindicated or not tolerated, and for the prophylaxis of migraine attacks, as warranted

In order to fulfil the obligation to submit the results of an imposed non-interventional PASS in accordance with Article 107q of Directive 2001/83/EC, Sanofi-Aventis Recherche & Développement, on behalf of the MAH's consortium, submitted on 17 July 2025 a PASS final study report v1.0 to the EMA for medicines containing substances related to valproate (sodium valproate, valproic acid, valproate semi-sodium, valpromide, valproate magnesium).

PRAC discussed the final study results. PRAC discussed the final study results in addition to the MAH's responses to the request for supplementary information (RSI).

Summary of recommendation(s) and conclusions

- Based on the review of the final report of the category 1 drug utilisation (DUS) study and taking into account the MAH position discussed in the context of the oral explanation held on 07 July 2026, PRAC concluded that no update of valproate aRMM or risk minimisation strategy is justified at this moment. Considering all available information on pregnancy prevention programme (PPP) compliance and exposed pregnancies, PRAC also concluded that further pharmacovigilance activities are justified to ensure continued monitoring of drug utilisation patterns in women of childbearing potential (WCBP) and valproate exposed pregnancies in the future. Therefore, the MAHs which have a risk management plan in place should submit to the relevant national competent authority (NCA) an updated RMP, via a variation procedure to remove the completed category 1 DUS extension study and to include a category 3 DUS extension continuation study, which is a long-term study (protocol) to ensure continuous monitoring of valproate utilisation in WCBP and valproate exposed pregnancy rates over time in various EU Member States.

7.4. Results of PASS imposed and non-imposed in the marketing authorisation(s)²³

See also Annex [17.4](#).

7.4.1. Brexpiprazole – RXULTI (CAP) – EMA/VR/0000340645

Applicant: Otsuka Pharmaceutical Netherlands B.V.

PRAC Rapporteur: Miroslava Gocova

Scope: Submission of an updated RMP version 3.0 in order to remove National Pregnancy Registry for Atypical Antipsychotics (NPRAA) listed as Category 3 study from the RMP.

Background

For background information on substance(s) and indication(s) of centrally authorised product(s) identified as 'CAP', see [Human medicine European public assessment report \(EPAR\)](#) on the EMA website.

As stated in the RMP of Rxulti (brexpiprazole), the MAH conducted a category 3 non-imposed non-interventional PASS in order to further characterise the list of missing information defined according to the subpopulation of patients excluded from the clinical trial e.g. pregnant women, children. The Rapporteur assessed the MAH's final study report.

Summary of advice

- Based on the available data, the MAH's responses to the RSI and the Rapporteur's review, PRAC considered that the ongoing variation assessing the final study report could be recommended for approval.
- PRAC agreed with the removal of the category 3 study from the RMP and to

²³ In accordance with Article 61a (6) of Regulation (EC) No 726/2004, in line with the revised variations regulation for any submission as of 4 August 2013

further monitor 'use in pregnancy' and 'use during breastfeeding' as safety concerns in PSURs.

7.4.2. Ofatumumab – KESIMPTA (CAP) – EMA/VR/0000315689

Applicant: Novartis Europharm Limited

PRAC Rapporteur: Amelia Cupelli

Scope: Update of section 4.6 'pregnancy' of the SmPC based on the final reports from Kesimpta Pregnancy Registry and the PRenancy outcomes Intensive Monitoring (PRIM) study.

Background

For background information on substance(s) and indication(s) of centrally authorised product(s) identified as 'CAP', see [Human medicine European public assessment report \(EPAR\)](#) on the EMA website.

As stated in the RMP of Kesimpta (ofatumumab), the MAH conducted a category 3 non-imposed non-interventional PASS (Kesimpta PRIM study) to estimate the proportion of congenital malformations associated with exposure to ofatumumab during pregnancy. The Rapporteur assessed the MAH's final study report in addition to the MAH's answers to the request for supplementary information (RSI).

Summary of advice

- Based on the available data, the MAH's responses to the RSI and the Rapporteur's review, PRAC considered that the ongoing variation assessing the final study report could be recommended for approval.
- PRAC agreed that the product information needs to be updated to reflect the results of the Kesimpta PRIM study and the Kesimpta Pregnancy Registry (SmPC section 4.6, no update to the package leaflet is needed).

7.5. Interim results and other post-authorisation measures for imposed and non-imposed studies

See Annex [17.5](#).

8. Renewals of the marketing authorisation, conditional renewal and annual reassessments

8.1. Annual reassessments of the marketing authorisation

See Annex [18.1](#).

8.2. Conditional renewals of the marketing authorisation

See Annex [18.2](#).

8.3. Renewals of the marketing authorisation

See Annex [18.3](#).

9. Product related pharmacovigilance inspections

9.1. List of planned pharmacovigilance inspections

None

9.2. Ongoing or concluded pharmacovigilance inspections

Disclosure of information on results of pharmacovigilance inspections could undermine the protection of the purpose of these inspections, investigations and audits. Therefore such information is not reported in the minutes.

None

9.3. Others

None

10. Other safety issues for discussion requested by the Member States, CHMP or the EMA

10.1.1. Botulinum toxin type A (NAP) - IE/H/xxxx/WS/380

Applicant(s): AbbVie Limited

PRAC Lead: Eamon O Murchu

Scope: PRAC consultation on a type II national variation to update the product information regarding the risk of iatrogenic botulism, at request of Ireland

Summary of PRAC advice

- PRAC agreed with the LMS assessment and concluded that it would not be appropriate to amend the product information to state that most cases of iatrogenic botulism have been reported following off-label use of botulinum toxin products, or pertained to the use of unlicensed products, as cases have been reported for licensed products both following on- and off-label use. Therefore, PRAC agreed that the wording adopted in September 2025, in the context of PSUSA procedure PSUSA/00000426/202412 should remain unchanged. PRAC also agreed that additional editorial changes of the package leaflet are acceptable, however extrapolation of these minor editorial revisions to other botulinum toxin A products that have already implemented the PRAC recommended wording is not considered necessary. In addition, where a PRAC recommendation has not already been made, PRAC concluded that the issue will be examined for other botulinum toxin A products (e.g. botulinum type A - haemagglutinin complex) as part of their next planned PSUR assessment.

11. Scientific advice procedures

Information related to this section cannot be released at the present time as it is deemed to contain commercially confidential information

12. Organisational, regulatory and methodological matters

12.1. Mandate and organisation of PRAC

12.1.1. Draft learning pharmacovigilance map and learning priorities for 2026-2028

PRAC lead: Ulla Wändel Liminga

The EMA Secretariat presented to PRAC the draft pharmacovigilance assessor map and the learning priorities for 2026-2028. A call for a Pharmacovigilance Map Owner was launched. The Pharmacovigilance Map Owner will contribute to the further development of the PRAC learnings map and help identify and make learning needs visible in preparation for the event planned for Q4 2026. The role will also support the promotion of learning opportunities and the alignment of future learning activities with PRAC's evolving needs. The PRAC members were invited to express their interest for this role by 03 September 2026.

12.1.2. PRAC membership

None

12.1.3. PRAC Rules of procedure - revision

The EMA Secretariat presented to PRAC the revised rules of procedure, in the context of broadening the scope of OPEN framework to enable PRAC's involvement in the assessment of procedures conducted under this framework. The revision also included minor amendments, mainly for alignment with other Committees rules of procedure. PRAC adopted the rules of procedure without further amendments.

12.1.4. Nominated proxy

Annalisa Capuano gave a proxy right to Maria Teresa Herdeiro for the entire meeting.

12.1.5. Scientific Committee Meetings – alternating face-to-face and virtual meetings schedule for 2027

PRAC Secretariat presented the final alternating face-to-face and virtual meetings schedule for 2027. PRAC noted the information.

12.2. Coordination with EMA Scientific Committees or CMDh-v

None

12.3. Coordination with EMA Working Parties/Working Groups/Drafting Groups

12.3.1. Areas of collaboration between PRAC and Methodology Working Party (MWP)

PRAC lead: Ulla Wändel Liminga, Liana Martirosyan, Carla Torre

This topic was postponed for the next PRAC plenary meeting.

12.4. Cooperation within the EU regulatory network

None

12.5. Cooperation with International Regulators

None

12.6. Contacts of PRAC with external parties and interaction with the Interested Parties to the Committee

None

12.7. PRAC work plan

None

12.8. Planning and reporting

12.8.1. Marketing authorisation applications (MAA) forecast for 2026 – planning update dated Q2 2026

The EMA Secretariat presented for information to PRAC a quarterly updated report on marketing authorisation applications (MAA) with eligibility request for centralised procedure planned for submission/submitted (the business 'pipeline') in Q2 2026. PRAC noted the information.

12.9. Pharmacovigilance audits and inspections

12.9.1. Pharmacovigilance systems and their quality systems

None

12.9.2. Pharmacovigilance inspections

None

12.9.3. Pharmacovigilance audits

None

12.10. Periodic safety update reports (PSURs) & Union reference date (EURD) list

12.10.1. Periodic safety update reports

None

12.10.2. Granularity and Periodicity Advisory Group (GPAG)

PRAC lead: Petar Mas

The EMA Secretariat provided PRAC with a brief update on the topics discussed at the latest GPAG meeting, including the merging of EURD list entries. PRAC took note of the information.

12.10.3. PSURs repository

None

12.10.4. Union reference date list – consultation on the draft list

In line with the criteria for plenary presentation of updates to the EURD List adopted by PRAC in December 2021, PRAC endorsed the draft revised EURD list, version *July 2026*, reflecting the PRAC's comments impacting on the data lock point (DLP) and PSUR submission frequencies of the substances/combinations. PRAC endorsed the newly allocated Rapporteurs for upcoming PSUSAs in accordance with the principles previously endorsed by PRAC (see PRAC minutes April 2013).

Post-meeting note: following the PRAC meeting of July 2026, the updated EURD list was adopted by CHMP and CMDh at their July 2026 meetings and published on the EMA website, see: [Home> Human Regulatory>Post-authorisation>Pharmacovigilance>Periodic safety update reports>> List of Union reference dates and frequency of submission of periodic safety update reports \(PSURs\)](#)

12.11. Signal management

12.12. Adverse drug reactions reporting and additional monitoring

12.12.1. Management and reporting of adverse reactions to medicinal products

None

12.12.2. Additional monitoring

None

12.12.3. List of products under additional monitoring – consultation on the draft list

PRAC was informed on the updates made to the list of products under additional monitoring.

Post-meeting note: The updated additional monitoring list was published on the EMA website, see: [Home>Human Regulatory>Post-authorisation>Pharmacovigilance>Medicines under additional monitoring>List of medicines under additional monitoring](#)

12.13. EudraVigilance database

12.13.1. Activities related to the confirmation of full functionality

None

12.14. Risk management plans and effectiveness of risk minimisations

12.14.1. Risk management systems

None

12.14.2. Tools, educational materials and effectiveness measurement of risk minimisations

None

12.15. Post-authorisation safety studies (PASS)

12.15.1. Post-authorisation Safety Studies – imposed PASS

None

12.15.2. Post-authorisation Safety Studies – non-imposed PASS

None

12.16. Community procedures

12.16.1. Referral procedures for safety reasons

None

12.17. Renewals, conditional renewals, annual reassessments

None

12.18. Risk communication and transparency

12.18.1. Public participation in pharmacovigilance

None

12.18.2. Safety communication

None

12.19. Continuous pharmacovigilance

12.19.1. Incident management

None

12.20. Impact of pharmacovigilance activities

12.20.1. Reflection paper on digital support to risk minimisation measures (RMMs)

PRAC lead: To be appointed

One of the activities and deliverables of the PRAC workplan for 2026 is to provide expert input for developing a reflection paper on digital tools supporting risk minimisation measures. The background work for this activity has been presented at the PRAC Strategic Review and Learning Meeting in Cyprus in May 2026.

Following this presentation to PRAC, the EMA Secretariat circulated a proposal regarding digital formats of risk minimisation measures and digital support tools as a basis for developing the reflection paper as per the workplan 2026 actions. PRAC members were invited to provide comments on the proposal, with a view to endorsing a way forward at a forthcoming PRAC plenary meeting.

12.21. Others

12.21.1. Artificial Intelligence (AI) in pharmacovigilance guidance development

PRAC lead: Jean-Michel Dogné, Milou-Daniel Drici

Following the update from March 2026, a survey was conducted among key stakeholders to help shape the scope of the guidance on the use of AI in pharmacovigilance. The EMA Secretariat presented to PRAC the outcome of the survey, a draft outline for the guidance, together with an update on the activities concerning AI regulatory guidance and related documents. The EMA Secretariat also presented the mandate and composition of the AI Guidance Coordination Group, together with an overview of the planning and drafting activities for the coordinated AI guidance roadmap. PRAC noted the information.

12.21.2. DARWIN EU® study - Association between varicella vaccines (V), measles-mumps-rubella- varicella vaccines (MMRV), and encephalitis in paediatric recipients

PRAC Lead: Jean-Michel Dogné

PRAC discussed the PRAC Sponsor's appraisal of the DARWIN EU® Study 'Association between varicella vaccines (V), measles-mumps-rubella- varicella vaccines (MMRV), and encephalitis in paediatric recipients' ([EUPAS1000000813](#)).

The study was requested following a safety signal regarding a possible association between varicella-containing vaccines (V/MMRV) and encephalitis in children and adolescents. Based on the study results, it was concluded that encephalitis after varicella vaccination was very rare. No association was observed between varicella-containing vaccination and encephalitis in the studied databases, and findings were robust across sensitivity analyses. Regarding

the association between varicella infection and the risk of encephalitis, the number of events was small and estimates were imprecise.

PRAC discussed and agreed with the PRAC Sponsor's appraisal and concluded that the study findings did not warrant any regulatory action.

12.21.3. Good pharmacovigilance practice (GVP) – mid-year update 2026

PRAC lead: Ulla Wändel Liminga

The EMA Secretariat informed the Committee in writing of the progress made on the revision of the GVP Modules and the priorities for next year. This information will also be reflected in the PRAC Work Plan 2027, which will be published on the EMA website following its adoption by PRAC.

12.21.4. Guideline on good pharmacovigilance practices (GVP) Module III – Pharmacovigilance inspections (Rev 2)

Following agreement of the pharmacovigilance inspectors working group (PhV IWG), the draft revision 2 of GVP Module III on pharmacovigilance inspections was circulated to PRAC for written adoption by 06 July 2026.

Post-meeting note: PRAC adopted the revision 2 of GVP Module III without further comments.

12.21.5. Process for real-world interrogation network (DARWIN EU®) studies conducted for PRAC and real world evidence (RWE) – update

PRAC Lead: Liana Martirosyan

The EMA Secretariat presented to PRAC an update on RWE and DARWIN EU activities, including a summary of the annual RWE report, information on ongoing studies, as well as the proposed process for real world data studies conducted for PRAC through DARWIN.

Post-meeting note: The final version of the process was shared with PRAC for comments by 11 September 2026.

12.21.6. Transition to IRIS for initial marketing authorisation applications and pre-submission activities - update

The EMA Secretariat presented to PRAC the upcoming engagement activities to support preparation of the transition to IRIS for initial marketing authorisation applications and pre-submission activities. PRAC noted the information.

13. Any other business

None

14. Annex I – Signals assessment and prioritisation²⁴

As per the agreed criteria for new signal(s), PRAC adopted without further plenary discussion the recommendation of the Rapporteur to request MAH(s) to submit a cumulative review following standard timetables²⁵.

14.1. New signals detected from EU spontaneous reporting systems and/or other sources

14.1.1. Atogepant - AQUIPTA (CAP)

Applicant: AbbVie Deutschland GmbH & Co. KG

PRAC Rapporteur: Rugile Pilviniene

Scope: Signal of insomnia

EPITT 20291 – New signal

Lead Member State: LT

14.1.2. Sacituzumab govitecan - TRODELVY (CAP)

Applicant: Gilead Sciences Ireland UC

PRAC Rapporteur: Bianca Mulder

Scope: Signal of interstitial lung disease

EPITT 20290 – New signal

Lead Member State: NL

14.1.3. Selpercatinib – RETSEVMO (CAP)

Applicant: Eli Lilly Nederland B.V.

PRAC Rapporteur: Bianca Mulder

Scope: Signal of lymphangiectasia intestinal

EPITT 20289 – New signal

Lead Member State: NL

14.2. Signals follow-up and prioritisation

None

²⁴ Each signal refers to a substance or therapeutic class. The route of marketing authorisation is indicated in brackets (CAP for Centrally Authorised Products; NAP for Nationally Authorised Products including products authorised via Mutual Recognition Procedures and Decentralised Procedure). Product names are listed for reference Centrally Authorised Products (CAP) only. PRAC recommendations will specify the products concerned in case of any regulatory action required

²⁵ Either MAH(s)'s submission within 60 days followed by a 60 day-timetable assessment or MAH's submission cumulative review within an ongoing or upcoming PSUR/PSUSA procedure (if the DLP is within 90 days), and no disagreement has been raised before the meeting

14.3. Variation procedure(s) resulting from signal evaluation

None

15. Annex I – Risk management plans

15.1. Medicines in the pre-authorisation phase

As per the agreed criteria, PRAC endorsed without further plenary discussion the conclusions of the Rapporteur on the assessment of the RMP for the medicine(s) mentioned below under evaluation for initial marketing authorisation application. Information on the medicines containing the active substance(s) listed below will be made available following the CHMP opinion on their marketing authorisation(s).

15.1.1. Pegfilgrastim - (CAP MAA) - EMEA/H/C/006937

Scope (pre D-60 phase): Treatment of neutropenia

15.1.2. Pertuzumab - (CAP MAA) - EMEA/H/C/006844

Scope (pre D-180 phase): Treatment of breast cancer in adults

15.1.3. Riociguat - (CAP MAA) - EMEA/H/C/006838

Scope (pre D-180 phase): Treatment of chronic thromboembolic pulmonary hypertension (CTEPH) in adults and treatment of pulmonary arterial hypertension (PAH) in adults and children from 6 years of age

15.1.4. Ruxolitinib - (CAP MAA) - EMEA/H/C/006791

Scope (pre D-180 phase): Treatment of myelofibrosis (MF) in adults, treatment of polycythaemia vera (PV) in adults and treatment of graft versus host disease (GvHD) in adults and children

15.1.5. Tofacitinib - (CAP MAA) - EMEA/H/C/006698

Scope (pre D-180 phase): Treatment of rheumatoid arthritis, psoriatic arthritis, ankylosing spondylitis, and ulcerative colitis in adults, and juvenile idiopathic arthritis (JIA) in children from 2 years old

15.2. Medicines in the post-authorisation phase – PRAC-led procedures

As per the agreed criteria, PRAC endorsed without further plenary discussion the conclusions of the Rapporteur on the assessment of the variation procedure for the medicine(s) mentioned below.

15.2.1. Cabozantinib – CABOMETYX (CAP); COMETRIQ (CAP) – EMA/VR/0000338165

Applicant: Ipsen Pharma

PRAC Rapporteur: Bianca Mulder

Scope: Submission of an updated RMP version 9.1 in order to propose the reclassification and removal of safety concerns.

15.2.2. Siponimod – MAYZENT (CAP) – EMA/VR/0000317924

Applicant: Novartis Europharm Limited

PRAC Rapporteur: Maria del Pilar Rayon

Scope: A grouped application consisting of:

Type II (C.I.13): Submission of the final report from study CBAF312A2006 listed as a category 3 study in the RMP. This is a survey conducted among healthcare professionals and Multiple Sclerosis patients/caregivers in selected European countries plus Canada to evaluate the knowledge required for the safe use of Mayzent (siponimod). The RMP version 8.0 has also been submitted.

Type IB (C.I.11): Submission of an updated RMP version 8.0 in order to update the siponimod exposure data in accordance with the results of the drug-drug interaction study CBAF312A02101 in alignment with the EMA/VR/0000255116 procedure.

15.3. Medicines in the post-authorisation phase – CHMP-led procedures

15.3.1. Alpelisib – PIQRAY (CAP) – EMA/VR/0000317159

Applicant: Novartis Europharm Limited

PRAC Rapporteur: Bianca Mulder

Scope: Extension of indication for PIQRAY in combination with fulvestrant for the treatment of postmenopausal women, and men, with hormone receptor (HR)-positive, human epidermal growth factor receptor 2 (HER2)-negative, locally advanced or metastatic breast cancer with a PIK3CA mutation after disease progression following an endocrine-based regimen; based on the primary analysis (DCO 15-Oct-2024) from the Phase III Study CBYL719C2303 (C2303, EPIK-B5). This is a Phase III, randomized, double-blind, placebo-controlled study of alpelisib (BYL719) in combination with fulvestrant for men and postmenopausal women with HR-positive, HER2-negative advanced breast cancer with PIK3CA mutation, who progressed on or after aromatase inhibitor and a CDK4/6 inhibitor. As a consequence, sections 4.2, 4.4, 4.8, 5.1 and 5.2 of the SmPC are updated. The Package Leaflet is updated in accordance. Version 10.0 of the RMP has also been submitted.

15.3.2. Angiotensin II – GIAPREZA (CAP) – EMA/VR/0000343583

Applicant: Paion Pharma GmbH

PRAC Rapporteur: Bianca Mulder

Scope: Submission of the study protocol (CRH06) in order to modify the completion date of the Post-Authorization Efficacy Study (CRH06 / ARTISTRY) listed as an obligation in the Annex II. This is a Phase 4, randomized, double-blind, placebo-controlled, parallel group, multi-center post-authorization efficacy clinical trial of angiotensin II in adult patients with refractory vasodilatory shock and severe acute kidney injury. The Annex II and RMP version 2.1 are updated accordingly.

15.3.3. [Asciminib – SCEMBLIX \(CAP\) – EMA/VR/0000342126](#)

Applicant: Novartis Europharm Limited

PRAC Rapporteur: Eva Jirsová

Scope: Update of section 4.4 of the SmPC in order to add a new warning on arterial occlusive events, based on a safety review. The Package Leaflet is updated accordingly. The RMP version 8.0 has also been submitted.

15.3.4. [Autologous CD34+ enriched cell fraction that contains CD34+ cells transduced with retroviral vector that encodes for the human ADA cDNA sequence – STRIMVELIS \(CAP\) – EMA/VR/0000347859](#)

Applicant: Fondazione Telethon Ets

PRAC Rapporteur: Liana Martirosyan

Scope: Quality

15.3.5. [Bictegravir / Emtricitabine / Tenofovir alafenamide – BIKTARVY \(CAP\) – EMA/VR/0000339784](#)

Applicant: Gilead Sciences Ireland Unlimited Company

PRAC Rapporteur: Liana Martirosyan

Scope: Submission of the final report from study GS-US-380-1474 listed as a category 3 study in the RMP. This is a Phase 2/3, Open-Label Study of the Pharmacokinetics, Safety, and Antiviral Activity of the GS-9883/Emtricitabine/Tenofovir Alafenamide (GS-9883/F/TAF) Fixed Dose Combination (FDC) in HIV-1 Infected Adolescents and Children. The RMP version 5.1 has also been submitted.

15.3.6. [Cabotegravir – VOCABRIA \(CAP\) – EMA/VR/0000332087](#)

Applicant: ViiV Healthcare B.V.

PRAC Rapporteur: Dennis Lex

Scope: Update of sections 4.4, 4.6 and 5.2 of the SmPC in order to update information on pregnancy based on interim results from study HPTN 084/084-01; this is phase 3 double blind safety and efficacy study of long-acting injectable cabotegravir compared to daily oral TDF/FTC for pre-exposure prophylaxis in HIV-uninfected women – Pregnancy Safety and PK Interim Analysis; the Package Leaflet is updated accordingly. The RMP version 6.0 has also been submitted. In addition, the MAH took the opportunity to bring the PI in line with the latest QRD template and to make typographical edits.

15.3.7. [Cabotegravir – APRETUDE \(CAP\) – EMA/VR/0000331993](#)

Applicant: ViiV Healthcare B.V.

PRAC Rapporteur: Dennis Lex

Scope: Update of sections 4.6 and 5.2 of the SmPC in order to update information and recommendations on pregnancy, based on interim results from the open label extension

(OLE) phase of the Phase 3 study HPTN 084 (study 201739) on the use of cabotegravir (CAB) for HIV-1 pre-exposure prophylaxis (PrEP) during pregnancy. The Package Leaflet is updated accordingly. The RMP version 2.0 has also been submitted. In addition, the MAH took the opportunity to introduce updates to the information on excipients in alignment with the excipient guideline and to introduce minor editorial and formatting changes to the PI.

15.3.8. Cemiplimab – LIBTAYO (CAP) – EMA/VR/0000347296

Applicant: Regeneron Ireland Designated Activity Company

PRAC Rapporteur: Bianca Mulder

Scope: Update of sections 4.2 and 6.6 of the SmPC in order to apply the same posology approved for the adjuvant treatment of patients with cutaneous squamous cell carcinoma (CSCC) (350mg every 3 weeks (Q3W) for the first 12 weeks of treatment, followed by a switch to 700mg every 6 weeks (Q6W)) as an alternative dosing regimen for all approved indications. In addition, this variation includes an update to the infusion time (15 – 30 minutes) for the currently approved 350mg Q3W dosing regimen. This is based on pharmacokinetic and modelling data. The Package Leaflet has been updated accordingly. The RMP version 5.1 has also been submitted. In addition, the MAH took the opportunity to introduce minor changes to the SmPC to include the E-number for the polysorbate 80 excipient (section 6.1) and to remove the nominal vial size (section 6.5) to allow flexibility for different vial sizes that may be sourced by approved manufacturers.

15.3.9. Dimethyl fumarate – TECFIDERA (CAP) – EMA/VR/0000320745

Applicant: Biogen Netherlands B.V.

PRAC Rapporteur: Dennis Lex

Scope: Submission of the final study results from 109MS306 (CONNECT) Part 2 listed as a category 3 study in the RMP; this is a phase 3 efficacy and safety study of BG00012 in pediatric subjects with relapsing-remitting multiple sclerosis (RRMS). The primary objective of Part 2 is to evaluate the long-term safety of BG00012 in subjects who completed Week 96 in Part 1 of Study 109MS306. The secondary objective of Part 2 is to describe the long-term multiple sclerosis outcomes of BG00012 in subjects who completed Week 96 in Part 1 of Study 109MS306. The RMP version 17.1 has also been submitted.

15.3.10. Dolutegravir / Lamivudine – DOVATO (CAP) – EMA/X/0000335457

Applicant: ViiV Healthcare B.V.

PRAC Rapporteur: David Olsen

Scope: Extension application to introduce a new pharmaceutical form (dispersible tablet) associated with a new strength (5 mg/30 mg) grouped with an extension of indication type II variation (C.6.a) to include treatment of children weighing at least 20 kg for Dovato based on results from Study 207742 and Study 205861. Study 207742 is a randomised non-inferiority trial with nested PK to assess DTG/3TC fixed dose formulations for the maintenance of virological suppression in children with HIV infection aged 2 to <15 years old: Intensive PK Substudy; and study 205861 is a Phase 3b, open label, single arm study in Antiretroviral Therapy (ART)-naïve adolescents. As a consequence, sections 4.1, 4.2, 4.8, 5.1 and 5.2 of

the SmPC are updated. The Package Leaflet and Labelling are updated in accordance. Version 7.0 of the RMP has also been submitted. In addition, the Marketing authorisation holder (MAH) took the opportunity to implement editorial changes and to update the PI in accordance with the latest EMA excipients guideline.

15.3.11. Eravacycline – XERAVA (CAP) – EMA/VR/0000343900

Applicant: Paion Pharma GmbH

PRAC Rapporteur: Adam Przybylkowski

Scope: Update of section 5.1 of the SmPC in order to update the list of pathogens susceptible to eravacycline, based on the final results from study NC-IHMA-2018-01, listed as a category 3 study in the RMP; this is a surveillance study of the activity of eravacycline against bacteria collected from hospitals in Asia / Pacific, Europe and the USA from 2018 to 2022. The RMP version 4.2 has also been submitted.

15.3.12. Etranacogene dezaparvovec – HEMGENIX (CAP) – EMA/VR/0000347644

Applicant: CSL Behring GmbH

PRAC Rapporteur: Bianca Mulder

Scope: A grouped application consisting of:

Type II (C.9.c) to update the protocol and Statistical Analysis Plan (SAP) of Study CSL222_4001, listed as a Specific Obligation in the Annex II of the Product Information. CSL222_4001 is an observational, post-authorisation, long-term follow-up study designed to evaluate the effectiveness and safety of HEMGENIX in the commercial setting. The Annex II and the RMP (version 1.1) are updated accordingly.

Type IB (C.9.b) to request a one-year extension to the due date of Specific Obligation 3 (SOB3) under Annex II.E of the conditional marketing authorisation.

15.3.13. Evolocumab – REPATHA (CAP) – EMA/VR/0000322435

Applicant: Amgen Europe B.V.

PRAC Rapporteur: Kimmo Jaakkola

Scope: Extension of indication to extend the indication for REPATHA to include adults at high risk for a first cardiovascular event, based on the final results from study 20170625 (VESALIUS); this is a Phase 3, double-blind, randomized, placebo-controlled, multicenter study to evaluate the impact of evolocumab on major cardiovascular events in patients at high cardiovascular risk without prior myocardial infarction or stroke. As a consequence, sections 4.1, 4.2, 4.8 and 5.1 of the SmPC are updated. The Package Leaflet is updated in accordance. Version 9.0 of the RMP has also been submitted. In addition, the Marketing authorisation holder (MAH) took the opportunity to update the list of local representatives in the Package Leaflet. Furthermore, some typographical errors were corrected, and the PI is brought in line with the latest QRD template version.

15.3.14. Faricimab – VABYSMO (CAP) – EMA/VR/0000308736

Applicant: Roche Registration GmbH

PRAC Rapporteur: Carla Torre

Scope: Update of sections 4.4 and 5.1 of the SmPC in order to update safety information, based on final results from study GR42691 (AVONELLE-X) listed as a category 3 study in the RMP. This was a multicenter, open-label extension study to evaluate the long-term safety and tolerability of faricimab in patients with neovascular Age-related Macular Degeneration (nAMD). The Package Leaflet is updated accordingly. The RMP version 8.0 has also been submitted. In addition, the MAH took the opportunity to introduce administrative and editorial changes to the PI, including to the Annex II, Labelling and to the list of local representatives in the Package Leaflet.

15.3.15. Fedratinib – INREBIC (CAP) – EMA/VR/0000324950

Applicant: Bristol-Myers Squibb Pharma EEIG

PRAC Rapporteur: Sonja Radowan

Scope: A grouped application consisting of:

C.4. Update of sections 4.4, 4.8, and 5.1 of the SmPC in order to update clinical pharmacology, efficacy and safety information based on final results from study FEDR MF 002 listed as a category 3 study in the RMP; this is a phase 3, multicenter, open-label, randomized study to evaluate the efficacy and safety of fedratinib compared to best available therapy in subjects with dipss-intermediate or high-risk primary myelofibrosis, post-polycythemia vera myelofibrosis, or post-essential thrombocythemia myelofibrosis and previously treated with ruxolitinib; the Package Leaflet is updated accordingly. The RMP version 4.0 has also been submitted.

C.3. Update of section 4.8 of the SmPC in order to add subdural hematoma to the list of adverse drug reactions (ADRs) following recommendation of PSUSA PSUSA/00010909/202508.

15.3.16. Glycopyrronium – SIALANAR (CAP) – EMA/X/0000287532

Applicant: Proveca Pharma Limited

PRAC Rapporteur: Zane Neikena

Scope: Quality

15.3.17. Insulin icodec / Semaglutide – KYINSU (CAP) – EMA/VR/0000322527

Applicant: Novo Nordisk A/S

PRAC Rapporteur: Petar Mas

Scope: Extension of indication to include treatment of adults with insufficiently controlled type 2 diabetes mellitus as an adjunct to diet and exercise for KYINSU, based on results from the Phase 3b study NN1535-4988 (COMBINE 4); this is a 40-week study comparing the

efficacy and safety of once weekly IcoSema and daily insulin glargine 100 units/mL in participants with type 2 diabetes inadequately controlled on oral anti-diabetic drugs. As a consequence, sections 4.1, 4.2, 4.4, 4.8 and 5.1 of the SmPC are updated. The Package Leaflet is updated in accordance. Version 1.1 of the RMP has also been submitted. In addition, the Marketing authorisation holder (MAH) took the opportunity to introduce minor editorial changes to the PI.

15.3.18. Lacosamide – LACOSAMIDE UCB (CAP); VIMPAT (CAP) – EMA/VR/0000321459

Applicant: UCB Pharma

PRAC Rapporteur: Karin Bolin

Scope: Update of sections 4.2, 4.8, 5.1 and 5.2 of the SmPC in order to update clinical information based on final results from study SP0968 and study EP0223. SP0968 was a phase 2/3, multicenter, open-label, randomized, active comparator study that evaluated the PK, efficacy, safety, and tolerability of lacosamide in neonatal study participants with repeated electroencephalographic neonatal seizures compared with an Active Comparator chosen based on standard of care per the local practice and treatment guidelines. EP0223 is a comparative study on long-term neurodevelopmental outcomes in neonates treated with lacosamide versus other antiseizure medications for neonatal seizures. The RMP version 18.0 has also been submitted. In addition, the MAH took the opportunity to update the list of local representatives in the Package Leaflet and to implement corrections in some local languages in both Vimpat and Lacosamide UCB Product Information.

15.3.19. Mavacamten – CAMZYOS (CAP) – EMA/VR/0000343906

Applicant: Bristol-Myers Squibb Pharma EEIG

PRAC Rapporteur: Kimmo Jaakkola

Scope: Submission of the final report from study CV0271148, listed as a category 3 study in the RMP. This is a meta-analysis to assess cardiovascular safety of mavacamten. RMP version 8.0 has also been submitted.

15.3.20. Nusinersen – SPINRAZA (CAP) – EMA/VR/0000343403

Applicant: Biogen Netherlands B.V.

PRAC Rapporteur: Karin Bolin

Scope: A grouped application to update sections 4.8, 5.1 and 5.2 of the SmPC in order to update clinical safety, efficacy, and immunogenicity information based on the results from study 232SM201 (NURTURE), listed as a PAES in the Annex II, and study 232SM404 (RESPOND). NURTURE is a Phase 2 open-label study to assess the efficacy, safety, tolerability, and pharmacokinetics of multiple doses of nusinersen delivered intrathecally to subjects with genetically diagnosed and presymptomatic spinal muscular atrophy, while RESPOND is a Phase 4 study of nusinersen among patients with spinal muscular atrophy who received onasemnogene abeparvovec. The Annex II is updated accordingly. The RMP version 14.1 has also been submitted.

15.3.21. Pembrolizumab – KEYTRUDA (CAP) – EMA/VR/0000340550

Applicant: Merck Sharp & Dohme B.V.

PRAC Rapporteur: Bianca Mulder

Scope: Extension of indication to include, in combination with chemotherapy with or without radiation, the adjuvant treatment of mismatch repair deficient (dMMR) endometrial cancer in adults who are at high risk of recurrence, for KEYTRUDA, based on results from the study KEYNOTE-B21; this is a phase 3, randomized, double-blind study of pembrolizumab versus placebo in combination with adjuvant chemotherapy with or without radiotherapy for the treatment of newly diagnosed high risk endometrial cancer after surgery with curative intent (KEYNOTE-B21 / ENGOT-en11 / GOG 3053). As a consequence, sections 4.1, 4.2 and 5.1 of the SmPC are updated. The Package Leaflet is updated in accordance. Version 55.1 of the RMP has also been submitted.

15.3.22. Pemigatinib – PEMAZYRE (CAP) – EMA/VR/0000343983

Applicant: Incyte Biosciences Distribution B.V.

PRAC Rapporteur: Bianca Mulder

Scope: A grouped application consisting of two Type II Variations, as follows:

C.4: Update of sections 4.4, 4.5, and 4.8 of the SmPC in order to update clinical information based on final results from study INCB 54828-302 (FIGHT-302), listed as a specific obligation in the Annex II; this is a phase 3, open-label, randomized, active-controlled, multicenter study comparing the efficacy and safety of pemigatinib vs. gemcitabine plus cisplatin chemotherapy in adults with unresectable or metastatic cholangiocarcinoma with fibroblast growth factor receptor 2 (FGFR2) rearrangement. The Annex II and Package Leaflet are updated accordingly.

C.9: Submission of the final report from study CIBI375A201, listed as a specific obligation in the Annex II of the Product Information. This is a phase 2 open-label, single-arm, multicenter study evaluating the efficacy and safety of pemigatinib in subjects with surgically unresectable and locally advanced, recurrent or metastatic cholangiocarcinoma with FGFR2 fusion or rearrangement who have failed at least one prior systemic therapy. The Annex II is updated accordingly.

The RMP version 4.2 has also been submitted.

15.3.23. Respiratory syncytial virus vaccine (bivalent, recombinant) – ABRYSCO (CAP) – EMA/VR/0000343072

Applicant: Pfizer Europe MA EEIG

PRAC Rapporteur: Liana Martirosyan

Scope: Update of sections 4.8 and 5.1 of the SmPC in order to update efficacy and safety information based on results from Phase 3 studies C3671032 and C3671013. The RMP version 2.1 has also been submitted.

15.3.24. Sacituzumab govitecan – TRODELVY (CAP) – EMA/VR/0000320818

Applicant: Gilead Sciences Ireland Unlimited Company

PRAC Rapporteur: Bianca Mulder

Scope: Extension of indication to include Trodelvy, in combination with pembrolizumab, for the treatment of adult patients with unresectable locally advanced or metastatic TNBC who have not received prior systemic therapy for metastatic disease and whose tumours express PD-L1 with a combined positive score (CPS) ≥ 10 , based on results from study GS-US-592-6173 (ASCENT-04), which is a phase 3 study of sacituzumab govitecan (IMMU-132) and Pembrolizumab versus treatment of physician's choice and Pembrolizumab in patients with previously untreated, locally advanced inoperable or metastatic triple-negative breast cancer, whose tumors express PD-L1. As a consequence, sections 4.1, 4.2, 4.4, 4.8, 5.1 and 5.2 of the SmPC are updated. The Package Leaflet is updated in accordance. Version 4.2 of the RMP has also been submitted.

15.3.25. Secukinumab – COSENTYX (CAP) – EMA/VR/0000340032

Applicant: Novartis Europharm Limited

PRAC Rapporteur: Maria Martinez Gonzalez

Scope: Extension of indication to include treatment of active moderate to severe hidradenitis suppurativa (HS) (acne inversa) in adolescents 12 years and older with an inadequate response to conventional systemic HS therapy for COSENTYX, based on modelling and extrapolation of existing data. As a consequence, sections 4.1, 4.2, 5.1 and 5.2 of the SmPC are updated. The Package Leaflet is updated in accordance. Version 14.0 of the RMP has also been submitted. In addition, the MAH took the opportunity to introduce minor editorial changes to the PI, including to the list of local representatives in the Package Leaflet.

15.3.26. Semaglutide – WEGOVY (CAP) – EMA/VR/0000327359

Applicant: Novo Nordisk A/S

PRAC Rapporteur: Karin Bolin

Scope: Update of sections 4.1, 4.2, 4.4, 4.8, 5.1 and 5.2 of the SmPC in order to reflect clinical results related to adults with overweight/obesity and metabolic dysfunction-associated steatohepatitis (MASH) based on interim results from phase 3a clinical study NN9931-4553 (ESSENCE) as well as three additional clinical trials NN9931-4381, NN9931-4296 and NN9931-4492 in adults with metabolic dysfunction-associated steatotic liver disease and/or MASH; supportive non-clinical results have also been submitted. The Package Leaflet is updated accordingly. The RMP version 10.2 has also been submitted.

15.3.27. Tafasitamab – MINJUVI (CAP) – EMA/VR/0000339938

Applicant: Incyte Biosciences Distribution B.V.

PRAC Rapporteur: Jenny-Maria Jönsson

Scope: Extension of indication for MINJUVI, in combination with lenalidomide and rituximab, cyclophosphamide, doxorubicin, vincristine, and prednisone (R-CHOP) to include treatment of

adult patients with previously untreated diffuse large B-cell lymphoma (DLBCL), or high-grade B-cell lymphoma (HGBL) who have high-intermediate or high-risk disease (International Prognostic Index score of 3 or greater), based on results from phase 3 study MOR208C310 (frontMIND) and results from phase 1b study MOR208C107. As a consequence, sections 4.1, 4.2, 4.4, 4.8 and 5.1 of the SmPC are updated. The Package Leaflet is also updated in accordance. A revised RMP version 4.1 is also being submitted. In addition, the MAH took the opportunity to implement editorial changes to the PI.

15.3.28. Talquetamab – TALVEY (CAP) – EMA/VR/0000339914

Applicant: Janssen Cilag International

PRAC Rapporteur: Barbara Kovacic Bytyqi

Scope: Extension of indication for TALVEY in combination with daratumumab and pomalidomide, or in combination with daratumumab, to include treatment of adult patients with relapsed or refractory multiple myeloma, who have received at least one prior therapy based on interim analysis from Phase 3 study MonumentAL-3 (64407564MMY3002) and data from the supportive Phase 1b study TriMM-2 (64407564MMY1002). As a consequence, sections 4.1, 4.2, 4.4, 4.8, 5.1, 5.2 and 6.6 of the SmPC are updated. The Package Leaflet is updated accordingly. The updated RMP version 4.1 has also been submitted. Consequently, the MAH proposes a switch from conditional marketing authorisation to full marketing authorisation. In addition, the MAH took the opportunity to update Annex II and to update the list of local representatives in the Package Leaflet. Furthermore, as part of the application the MAH is requesting a 1-year extension of the market protection.

15.3.29. Tucatinib – TUKYSA (CAP) – EMA/VR/0000341660

Applicant: Pfizer Europe MA EEIG

PRAC Rapporteur: Jean-Michel Dogné

Scope: Update of sections 4.4, and 4.8 of the SmPC in order to update information concerning hepatotoxicity based on the review of aggregate data from multiple studies, including randomized, controlled studies (HER2CLIMB-05, HER2CLIMB, and HER2CLIMB-02), and study MK-7119-001. The RMP version 4.0 has also been submitted.

15.3.30. Upadacitinib – RINVOQ (CAP) – EMA/X/0000304823

Applicant: Abbvie Deutschland GmbH & Co. KG

PRAC Rapporteur: Petar Mas

Scope: Extension application to introduce a new pharmaceutical form associated with a new strength and change of pharmacokinetics (1 mg/ml oral solution) grouped with an extension of indication (C.I.6.a) to include the treatment of active polyarticular course juvenile idiopathic arthritis (pcJIA) in patients 2 years of age and older based on clinical data and results from clinical phase 1 study (study M15-340). As a consequence, sections 4.1, 4.2, 4.5, 4.8, 5.1, 5.2 and 5.3 of the SmPC have been updated. The Package Leaflet has been updated accordingly. Version 17 of the RMP has also been submitted. In addition, the MAH took the opportunity to update Annex II.

15.3.31. Vamorolone – AGAMREE (CAP) – EMA/VR/0000343140

Applicant: Santhera Pharmaceuticals (Deutschland) GmbH

PRAC Rapporteur: Rhea Fitzgerald

Scope: Submission of updated comprehensive safety data from studies VBP15-006, SNT-I-VAM-024, SNT-I-VAM-025, and SNT-I-VAM-026, expanded-access programs, compassionate use and named patient programs as well as data from ongoing clinical trial SNT-IV-VAM-011 and post-marketing data. The RMP version 3.0 has also been submitted.

15.3.32. Vedolizumab – ENTYVIO (CAP) – EMA/VR/0000340504

Applicant: Takeda Pharma A/S

PRAC Rapporteur: Adam Przybylkowski

Scope: A grouped application consisting of:

Two Type II - C.6.a: Extension of indication to include treatment of ulcerative colitis (UC) and of Crohn's disease (CD) in the paediatric population from 2 to less than 18 years for Entyvio 300 mg powder for concentrate for solution for infusion (intravenous, IV), based on final results from studies MLN0002-2005, vedolizumab-2005 and MLN002-3024 and interim results from studies MLN0002-3025 and MLN0002-3029. These are phase 2 and phase 3 interventional studies to assess the pharmacokinetics, efficacy and safety of vedolizumab intravenous in paediatric UC and CD. As a consequence, sections 4.1, 4.2, 4.4, 4.5, 4.8, 5.1, 5.2, 5.3, 6.3 and 6.6 of the IV SmPC and Sections 4.2 and 5.1 of the subcutaneous (SC) product SmPC are updated. The IV Package Leaflet (PL) is updated in accordance. In addition, the MAH took the opportunity to implement minor editorial changes in the PI, and to update the list of local representatives in the PL. Version 10.0 of the RMP has also been submitted

Type II – C.4: To update section 6.6. of the SmPC to add a 50 mL saline bag for dilution prior to administration for the finished product. The PL has been updated accordingly.

16. Annex I - Periodic safety update reports (PSURs)

Based on the assessment of the following PSURs, PRAC concluded that the benefit-risk balance of the medicine(s) mentioned below remains favourable in the approved indication(s) and adopted a recommendation to maintain the current terms of the marketing authorisation(s) together with the assessment report. As per the agreed criteria, the procedures listed below were finalised at the PRAC level without further plenary discussion.

The next PSURs should be submitted in accordance with the requirements set out in the list of Union reference dates (EURD list) provided for under Article 107c(7) of Directive 2001/83/EC and published on the European medicines web-portal, unless changes apply as stated in the outcome of the relevant PSUR/PSUSA procedure(s).

16.1. PSUR single assessment (PSUSA) procedures including centrally authorised products (CAPs) only

16.1.1. Acridinium / Formoterol fumarate dihydrate – BRIMICA GENUAIR (CAP); DUAKLIR GENUAIR (CAP) – EMA/PSUR/0000336119

Applicant: Covis Pharma Europe B.V.

PRAC Rapporteur: Adam Przybylkowski

Scope: Evaluation of a PSUSA procedure (PSUSA/00010307/202511)

16.1.2. Adagrasib – KRAZATI (CAP) – EMA/PSUR/0000336084

Applicant: Bristol-Myers Squibb Pharma EEIG

PRAC Rapporteur: Kimmo Jaakkola

Scope: Evaluation of a PSUSA procedure (PSUSA/00000214/202512)

16.1.3. Arpraziquantel – ARPRAZIQUEL (Art 58) – EMA/PSUR/0000333585

Applicant: Merck Europe B.V.

PRAC Rapporteur: Adam Przybylkowski

Scope: Evaluation of a PSUSA procedure (PSUV arpraziquantel)

16.1.4. Autologous CD34+ enriched cell fraction that contains CD34+ cells transduced with retroviral vector that encodes for the human ADA cDNA sequence – STRIMVELIS (CAP) – EMA/PSUR/0000336165

Applicant: Fondazione Telethon Ets

PRAC Rapporteur: Liana Martirosyan

Scope: Evaluation of a PSUSA procedure (PSUSA/00010505/202511)

16.1.5. Benralizumab – FASENRA (CAP) – EMA/PSUR/0000336112

Applicant: AstraZeneca AB

PRAC Rapporteur: David Olsen

Scope: Evaluation of a PSUSA procedure (PSUSA/00010661/202511)

16.1.6. Blinatumomab – BLINCYTO (CAP) – EMA/PSUR/0000336108

Applicant: Amgen Europe B.V.

PRAC Rapporteur: Veronika Macurova

Scope: Evaluation of a PSUSA procedure (PSUSA/00010460/202512)

16.1.7. Budesonide – KINPEYGO (CAP) – EMA/PSUR/0000336136

Applicant: STADA Arzneimittel AG

PRAC Rapporteur: Marie Louise Schougaard Christiansen

Scope: Evaluation of a PSUSA procedure (PSUSA/00011007/202512)

16.1.8. Concentrate of proteolytic enzymes enriched in bromelain – NEXOBRID (CAP) – EMA/PSUR/0000336099

Applicant: MediWound Germany GmbH

PRAC Rapporteur: Dennis Lex

Scope: Evaluation of a PSUSA procedure (PSUSA/00010028/202512)

16.1.9. COVID-19 vaccine (recombinant, adjuvanted) – NUVAXOVID (CAP) – EMA/PSUR/0000336124

Applicant: Sanofi Winthrop Industrie

PRAC Rapporteur: Dirk Mentzer

Scope: Evaluation of a PSUSA procedure (PSUSA/00010972/202512)

16.1.10. Datopotamab deruxtecan – DATROWAY (CAP) – EMA/PSUR/0000336154

Applicant: Daiichi Sankyo Europe GmbH

PRAC Rapporteur: Jenny-Maria Jönsson

Scope: Evaluation of a PSUSA procedure (PSUSA/00011129/202512)

16.1.11. Deutivacaftor / Tezacaftor / Vanzacaftor – ALYFTREK (CAP) – EMA/PSUR/0000336139

Applicant: Vertex Pharmaceuticals (Ireland) Limited

PRAC Rapporteur: Dennis Lex

Scope: Evaluation of a PSUSA procedure (PSUSA/00011146/202512)

16.1.12. Efgartigimod alfa – VYVGART (CAP) – EMA/PSUR/0000336133

Applicant: Argenx

PRAC Rapporteur: Rhea Fitzgerald

Scope: Evaluation of a PSUSA procedure (PSUSA/00011014/202512)

16.1.13. Elacestrant – ORSERDU (CAP) – EMA/PSUR/0000336096

Applicant: Stemline Therapeutics B.V.

PRAC Rapporteur: Sonja Radowan

Scope: Evaluation of a PSUSA procedure (PSUSA/00000120/202512)

[16.1.14. Elafibranor – IQIRVO \(CAP\) – EMA/PSUR/0000336148](#)

Applicant: Ipsen Pharma

PRAC Rapporteur: Rugile Pilviniene

Scope: Evaluation of a PSUSA procedure (PSUSA/00011092/202512)

[16.1.15. Entrectinib – ROZLYTREK \(CAP\) – EMA/PSUR/0000336153](#)

Applicant: Roche Registration GmbH

PRAC Rapporteur: Bianca Mulder

Scope: Evaluation of a PSUSA procedure (PSUSA/00010874/202512)

[16.1.16. Eplontersen – WAINZUA \(CAP\) – EMA/PSUR/0000336131](#)

Applicant: AstraZeneca AB

PRAC Rapporteur: Jan Neuhauser

Scope: Evaluation of a PSUSA procedure (PSUSA/00011120/202512)

[16.1.17. Ferric citrate coordination complex – XOANACYL \(CAP\) – EMA/PSUR/0000336132](#)

Applicant: Averoa

PRAC Rapporteur: Kimmo Jaakkola

Scope: Evaluation of a PSUSA procedure (PSUSA/00011140/202512)

[16.1.18. Flortaucipir \(18F\) – TAUVID \(CAP\) – EMA/PSUR/0000336149](#)

Applicant: Eli Lilly Nederland B.V.

PRAC Rapporteur: Adam Przybylowski

Scope: Evaluation of a PSUSA procedure (PSUSA/00011073/202511)

[16.1.19. Givinostat – DUVYZAT \(CAP\) – EMA/PSUR/0000336164](#)

Applicant: Italfarmaco S.p.A.

PRAC Rapporteur: Liana Martirosyan

Scope: Evaluation of a PSUSA procedure (PSUSA/00011141/202512)

[16.1.20. Human papillomavirus vaccine \[types 16, 18\] \(recombinant, adjuvanted, adsorbed\) – CERVARIX \(CAP\) – EMA/PSUR/0000336113](#)

Applicant: GlaxoSmithKline Biologicals

PRAC Rapporteur: Jean-Michel Dogné

Scope: Evaluation of a PSUSA procedure (PSUSA/00009175/202511)

[16.1.21. Hydroxocobalamin – CYANOKIT \(CAP\) – EMA/PSUR/0000336100](#)

Applicant: Serb

PRAC Rapporteur: Zoubida Amimour

Scope: Evaluation of a PSUSA procedure (PSUSA/00010228/202511)

[16.1.22. Imetelstat – RYTELO \(CAP\) – EMA/PSUR/0000336130](#)

Applicant: Geron Netherlands B.V.

PRAC Rapporteur: Bianca Mulder

Scope: Evaluation of a PSUSA procedure (PSUSA/00011117/202512)

[16.1.23. Inclisiran – LEQVIO \(CAP\) – EMA/PSUR/0000336155](#)

Applicant: Novartis Europharm Limited

PRAC Rapporteur: Kimmo Jaakkola

Scope: Evaluation of a PSUSA procedure (PSUSA/00010904/202512)

[16.1.24. Inotuzumab ozogamicin – BESPONSA \(CAP\) – EMA/PSUR/0000336115](#)

Applicant: Pfizer Europe MA EEIG

PRAC Rapporteur: Dirk Mentzer

Scope: Evaluation of a PSUSA procedure (PSUSA/00010659/202512)

[16.1.25. Iptacopan – FABHALTA \(CAP\) – EMA/PSUR/0000336141](#)

Applicant: Novartis Europharm Limited

PRAC Rapporteur: Lina Seibokiene

Scope: Evaluation of a PSUSA procedure (PSUSA/00011054/202512)

[16.1.26. Ixazomib – NINLARO \(CAP\) – EMA/PSUR/0000336109](#)

Applicant: Takeda Pharma A/S

PRAC Rapporteur: Jenny-Maria Jönsson

Scope: Evaluation of a PSUSA procedure (PSUSA/00010535/202511)

[16.1.27. Levodopa – INBRIJA \(CAP\) – EMA/PSUR/0000336140](#)

Applicant: Merz Therapeutics GmbH

PRAC Rapporteur: Barbara Kovacic Bytyqi

Scope: Evaluation of a PSUSA procedure (PSUSA/00107800/202512)

[16.1.28. Nirmatrelvir / Ritonavir – PAXLOVID \(CAP\) – EMA/PSUR/0000336147](#)

Applicant: Pfizer Europe MA EEIG

PRAC Rapporteur: Dennis Lex

Scope: Evaluation of a PSUSA procedure (PSUSA/00010984/202512)

[16.1.29. Octreotide – OCZYESA \(CAP\) – EMA/PSUR/0000336143](#)

Applicant: Camurus AB

PRAC Rapporteur: Eamon O Murchu

Scope: Evaluation of a PSUSA procedure (PSUSA/00011036/202512)

[16.1.30. Olezarsen – TRYNGOLZA \(CAP\) – EMA/PSUR/0000336151](#)

Applicant: Swedish Orphan Biovitrum AB (publ)

PRAC Rapporteur: Kimmo Jaakkola

Scope: Evaluation of a PSUSA procedure (PSUSA/00011155/202512)

[16.1.31. Padeliporfin – TOOKAD \(CAP\) – EMA/PSUR/0000336122](#)

Applicant: Steba Biotech S.A.

PRAC Rapporteur: Maia Uusküla

Scope: Evaluation of a PSUSA procedure (PSUSA/00010654/202511)

[16.1.32. Pegvisomant – SOMAVERT \(CAP\) – EMA/PSUR/0000336093](#)

Applicant: Pfizer Europe MA EEIG

PRAC Rapporteur: Zoubida Amimour

Scope: Evaluation of a PSUSA procedure (PSUSA/00002328/202511)

[16.1.33. Pegzilarginase – LOARGYS \(CAP\) – EMA/PSUR/0000336083](#)

Applicant: Immedica Pharma AB

PRAC Rapporteur: Dennis Lex

Scope: Evaluation of a PSUSA procedure (PSUSA/00000222/202512)

[16.1.34. Pneumococcal polysaccharide conjugate vaccine \(20-valent, adsorbed\) – PREVENAR 20 \(CAP\) – EMA/PSUR/0000336138](#)

Applicant: Pfizer Europe MA EEIG

PRAC Rapporteur: Jean-Michel Dogné

Scope: Evaluation of a PSUSA procedure (PSUSA/00010981/202512)

16.1.35. Pneumococcal polysaccharide conjugate vaccine (21-valent) – CAPVAXIVE (CAP) – EMA/PSUR/0000336152

Applicant: Merck Sharp & Dohme B.V.

PRAC Rapporteur: Jean-Michel Dogné

Scope: Evaluation of a PSUSA procedure (PSUSA/00011121/202512)

16.1.36. Prasterone – INTRAROSA (CAP) – EMA/PSUR/0000336121

Applicant: Endoceutics S.A.

PRAC Rapporteur: Bianca Mulder

Scope: Evaluation of a PSUSA procedure (PSUSA/00010672/202511)

16.1.37. Respiratory syncytial virus mRNA vaccine (nucleoside modified) – MRESVIA (CAP) – EMA/PSUR/0000336146

Applicant: Moderna Biotech Spain S.L.

PRAC Rapporteur: Jean-Michel Dogné

Scope: Evaluation of a PSUSA procedure (PSUSA/00011075/202511)

16.1.38. Sepiapterin – SEPHIENCE (CAP) – EMA/PSUR/0000336127

Applicant: PTC Therapeutics International Limited

PRAC Rapporteur: David Olsen

Scope: Evaluation of a PSUSA procedure (PSUSA/00011145/202512)

16.1.39. Sipavibart – KAVIGALE (CAP) – EMA/PSUR/0000336123

Applicant: AstraZeneca AB

PRAC Rapporteur: Kimmo Jaakkola

Scope: Evaluation of a PSUSA procedure (PSUSA/00011113/202512)

16.1.40. Susoctocog alfa – OBIZUR (CAP) – EMA/PSUR/0000336105

Applicant: BAXALTA INNOVATIONS GmbH

PRAC Rapporteur: Dirk Mentzer

Scope: Evaluation of a PSUSA procedure (PSUSA/00010458/202511)

16.1.41. Tezepelumab – TEZSPIRE (CAP) – EMA/PSUR/0000336135

Applicant: AstraZeneca AB

PRAC Rapporteur: Eva Jirsová

Scope: Evaluation of a PSUSA procedure (PSUSA/00011015/202512)

[16.1.42. Tirbanibulin – KLISYRI \(CAP\) – EMA/PSUR/0000336129](#)

Applicant: Almirall S.A.

PRAC Rapporteur: Jana Pecherova

Scope: Evaluation of a PSUSA procedure (PSUSA/00010943/202512)

[16.1.43. Toripalimab – LOQTORZI \(CAP\) – EMA/PSUR/0000336150](#)

Applicant: Topalliance Biosciences Europe Limited

PRAC Rapporteur: Karin Erneholm

Scope: Evaluation of a PSUSA procedure (PSUSA/00011094/202512)

[16.1.44. Tralokinumab – ADTRALZA \(CAP\) – EMA/PSUR/0000336125](#)

Applicant: LEO PHARMA A/S

PRAC Rapporteur: Kimmo Jaakkola

Scope: Evaluation of a PSUSA procedure (PSUSA/00010937/202512)

[16.1.45. Trastuzumab deruxtecan – ENHERTU \(CAP\) – EMA/PSUR/0000336128](#)

Applicant: Daiichi Sankyo Europe GmbH

PRAC Rapporteur: Carla Torre

Scope: Evaluation of a PSUSA procedure (PSUSA/00010894/202512)

[16.1.46. Ublituximab – BRIUMVI \(CAP\) – EMA/PSUR/0000336163](#)

Applicant: Neuraxpharm Pharmaceuticals S.L.

PRAC Rapporteur: Liana Martirosyan

Scope: Evaluation of a PSUSA procedure (PSUSA/00000045/202512)

[16.1.47. Vadadustat – VAFSEO \(CAP\) – EMA/PSUR/0000336137](#)

Applicant: Medice Arzneimittel Puetter GmbH & Co. KG

PRAC Rapporteur: Eva Jirsová

Scope: Evaluation of a PSUSA procedure (PSUSA/00011050/202512)

16.2. PSUR single assessment (PSUSA) procedures including centrally authorised products (CAPs) and nationally authorised products (NAPs)

16.2.1. Clopidogrel – CLOPIDOGREL ZENTIVA (CAP); ISCOVER (CAP); PLAVIX (CAP); Clopidogrel / Acetylsalicylic acid – DUOPLAVIN (CAP); NAP – EMA/PSUR/0000336095

Applicants: Sanofi Winthrop Industrie, Zentiva k.s., various

PRAC Rapporteur: Carla Torre

Scope: Evaluation of a PSUSA procedure (PSUSA/00000820/202511)

16.2.2. Edotreotide – SOMAKIT TOC (CAP); NAP – EMA/PSUR/0000336114

Applicants: Advanced Accelerator Applications, various

PRAC Rapporteur: Rhea Fitzgerald

Scope: Evaluation of a PSUSA procedure (PSUSA/00010552/202512)

16.3. PSUR single assessment (PSUSA) procedures including nationally authorised products (NAPs) only

16.3.1. Apomorphine – EMA/PSUR/0000336097

Applicants: various

PRAC Lead: Karin Erneholm

Scope: Evaluation of a PSUSA procedure (PSUSA/00000227/202511)

16.3.2. Ascorbic acid / rutoside, ascorbic acid / rutoside / alfatocopherol – EMA/PSUR/0000336104

Applicants: various

PRAC Lead: Adam Przybylkowski

Scope: Evaluation of a PSUSA procedure (PSUSA/00010504/202512)

16.3.3. Benazepril – EMA/PSUR/0000336080

Applicants: various

PRAC Lead: Amelia Cupelli

Scope: Evaluation of a PSUSA procedure (PSUSA/00000313/202511)

16.3.4. Benzalkonium / benzocaine / tyrothricin – EMA/PSUR/0000336111

Applicants: various

PRAC Lead: Dennis Lex

Scope: Evaluation of a PSUSA procedure (PSUSA/00003070/202512)

16.3.5. Benzalkonium chloride / isopropyl myristate / paraffin liquid, benzalkonium chloride / chlorhexidine / isopropyl myristate / paraffin liquid – EMA/PSUR/0000336117

Applicants: various

PRAC Lead: Rhea Fitzgerald

Scope: Evaluation of a PSUSA procedure (PSUSA/00010640/202512)

16.3.6. Biclotymol / enoxolone / muramidase hydrochloride – EMA/PSUR/0000336089

Applicants: various

PRAC Lead: Guðrún Stefánsdóttir

Scope: Evaluation of a PSUSA procedure (PSUSA/00000409/202512)

16.3.7. Brotizolam – EMA/PSUR/0000336092

Applicants: various

PRAC Lead: Ana Sofia Diniz Martins

Scope: Evaluation of a PSUSA procedure (PSUSA/00000444/202512)

16.3.8. Caffeine / ergotamine – EMA/PSUR/0000336078

Applicants: various

PRAC Lead: Tiphaine Vaillant

Scope: Evaluation of a PSUSA procedure (PSUSA/00000485/202511)

16.3.9. Caffeine / paracetamol / propyphenazone, paracetamol / propyphenazone – EMA/PSUR/0000336094

Applicants: various

PRAC Lead: Roxana Stefania Udrescu

Scope: Evaluation of a PSUSA procedure (PSUSA/00002298/202512)

16.3.10. Carbaethopendecinium bromide / trimecaine hydrochloride – EMA/PSUR/0000336106

Applicants: various

PRAC Lead: Eva Jirsová

Scope: Evaluation of a PSUSA procedure (PSUSA/00003041/202512)

16.3.11. Chlorhexidine gluconate / isopropyl alcohol (covers other synonymous terms such as 2-propanol) – EMA/PSUR/0000336079

Applicants: various

PRAC Lead: Jo Robays

Scope: Evaluation of a PSUSA procedure (PSUSA/00000669/202512)

16.3.12. Clomifene – EMA/PSUR/0000336082

Applicants: various

PRAC Lead: Eva Jirsová

Scope: Evaluation of a PSUSA procedure (PSUSA/00000810/202512)

16.3.13. Dienogest – EMA/PSUR/0000336102

Applicants: various

PRAC Lead: Bianca Mulder

Scope: Evaluation of a PSUSA procedure (PSUSA/00003167/202512)

16.3.14. Idarubicin – EMA/PSUR/0000336098

Applicants: various

PRAC Lead: Jan Neuhauser

Scope: Evaluation of a PSUSA procedure (PSUSA/00001720/202511)

16.3.15. Indapamide – EMA/PSUR/0000336085

Applicants: various

PRAC Lead: Dennis Lex

Scope: Evaluation of a PSUSA procedure (PSUSA/00001731/202511)

16.3.16. Naltrexone – EMA/PSUR/0000336088

Applicants: various

PRAC Lead: Eamon O Murchu

Scope: Evaluation of a PSUSA procedure (PSUSA/00002117/202511)

16.3.17. Pancuronium – EMA/PSUR/0000336090

Applicants: various

PRAC Lead: Dennis Lex

Scope: Evaluation of a PSUSA procedure (PSUSA/00002275/202512)

16.3.18. Sultamicillin – EMA/PSUR/0000336101

Applicants: various

PRAC Lead: Maia Uusküla

Scope: Evaluation of a PSUSA procedure (PSUSA/00002829/202511)

16.3.19. Vecuronium bromide – EMA/PSUR/0000336110

Applicants: various

PRAC Lead: Eva Jirsová

Scope: Evaluation of a PSUSA procedure (PSUSA/00003102/202511)

16.3.20. Yellow fever vaccine (live) – EMA/PSUR/0000337795

Applicants: various

PRAC Lead: Dirk Mentzer

Scope: Evaluation of a PSUSA procedure (PSUSA/00003135/202512)

16.4. Follow-up to PSUR/PSUSA procedures

16.4.1. Rotavirus vaccine, live (NAP) – EMA/PAM/0000343686

Applicants: various

PRAC Rapporteur: Jean-Michel Dogné

Scope: LEG - a legally binding cumulative safety reviews of all serious cases of rotavirus gastroenteritis in children without immune deficiency, of vomiting and of immune thrombocytopenic purpura (ITP) requested by PRAC further to the assessment of the latest PSUR.

Considering the findings an update of the product information and/or the risk management plan could be warranted.

16.5. Variation procedure(s) resulting from PSUSA evaluation

16.5.1. Zilucoplan – ZILBRYSQ (CAP) – EMA/VR/0000343399

Applicant: UCB Pharma

PRAC Rapporteur: Karin Erneholm

Scope: Update of Annex IID of the PI in order to implement the outcome of PSUR procedure, PSUSA/00000169/202509, recommending to remove the Controlled Access Program linked to the safety concern 'meningococcal infection' from the RMP. The RMP version v1.1 has also been submitted along with updated educational materials, as per PRAC recommendation.

16.6. Expedited summary safety reviews²⁶

None

17. Annex I – Post-authorisation safety studies (PASS)

Based on the assessment of the following PASS protocol(s), result(s), interim result(s) or feasibility study(ies), and following endorsement of the comments received, PRAC adopted the conclusion of the Rapporteurs on their assessment for the medicines listed below without further plenary discussion.

17.1. Protocols of PASS imposed in the marketing authorisation(s)²⁷

17.1.1. Beremagene geperpavec – VYJUVEK (CAP) – EMA/PASS/0000287685

Applicant: Krystal Biotech Netherlands B.V.

PRAC Rapporteur: Liana Martirosyan

Scope: PASS protocol [107n]: A prospective, non-interventional, multi-country study to confirm the long-term safety profile, including in paediatric patients less than 6 months of age, of B-VEC for the treatment of DEB wounds in a real-life clinical setting.

17.2. Protocols of PASS non-imposed in the marketing authorisation(s)²⁸

17.2.1. Cabotegravir – APRETUDE (CAP) – EMA/PAM/0000343607

Applicant: ViiV Healthcare B.V.

PRAC Rapporteur: Dennis Lex

Scope: substantial amendments to the protocol of the following Apretude PAM: "CAB LA PrEP Cohort: Prospective Cohort Study to Assess Adherence and Effectiveness of, and Monitor for Hepatotoxicity and Resistance to, Cabotegravir for Pre-Exposure Prophylaxis in Europe" (PASS Study, RMP cat 3).

17.2.2. Chikungunya vaccine (live) – IXCHIQ (CAP) – EMA/PAM/0000341603

Applicant: Valneva Austria GmbH

PRAC Rapporteur: Dirk Mentzer

Scope: Submission of the protocols for the the post-authorisation safety studies VLA1553-403 (version 6.0) and VLA1553-406 (version 6.0) which are category 3 studies in the RMP. VLA1553-403 is an observational study to evaluate the safety of VLA1553 in pregnant women exposed to th vaccine In Brazil. VLA1553-406 is a prospective safety cohort study.

²⁶ Submission of expedited summary safety reports for review in addition to the requirements for submission of PSUR(s) falling within the pandemic period and requirements set out in the list of Union reference dates (EURD list) provided for under Article 107c(7) of Directive 2001/83/EC

²⁷ In accordance with Article 107n of Directive 2001/83/EC

²⁸ In accordance with Article 107m of Directive 2001/83/EC, supervised by PRAC in accordance with Article 61a (6) of Regulation (EC) No 726/2004

17.2.3. Cladribine – MAVENCLAD (CAP) – EMA/PAM/0000319565

Applicant: Merck Europe B.V.

PRAC Rapporteur: Carla Torre

Scope: Protocol amendment for PASS MS 700568-0002: Long-term prospective, observational cohort study evaluating the safety profile, in terms of incidence of adverse events of special interest, in patients with highly active relapsing multiple sclerosis (RMS) newly started on oral cladribine (CLARION)

17.2.4. Inebilizumab – UPLIZNA (CAP) – EMA/PAM/0000325493

Applicant: Amgen Europe B.V.

PRAC Rapporteur: Amelia Cupelli

Scope: Protocol amendment submission of PASS Cat.3 Study A real-world observational study of treatment patterns and outcomes for patients with neuromyelitis optica spectrum disorders (NMOSDs) and immunoglobulin G4-related disease (IgG4-RD) treated with inebilizumab (UPLIZNA) in Europe

17.2.5. Levacetylleucine – AQNEURSA (CAP) – EMA/PAM/0000340768

Applicant: Intrabio Ireland Limited

PRAC Rapporteur: Dennis Lex

Scope: Protocol submission for Post-authorisation, non-interventional study using existing International Niemann Pick Disease Registry (INPDR) as data source

17.2.6. Maribavir – LIVTENCITY (CAP) – EMA/PAM/0000343616

Applicant: Takeda Pharmaceuticals International AG Ireland Branch

PRAC Rapporteur: Adam Przybylowski

Scope: Submission of a second protocol amendment for Study TAK-620-4007, a Retrospective chart review of safety outcomes associated with use of maribavir in patients with post-transplant refractory cytomegalovirus (CMV) infection and comorbid severe chronic kidney disease (CKD) or comorbid end-stage renal disease (ESRD), including patients on peritoneal dialysis or hemodialysis.

17.2.7. Rimegepant – VYDURA (CAP) – EMA/PAM/0000343596

Applicant: Pfizer Europe MA EEIG

PRAC Rapporteur: Karin Erneholm

Scope: Fifth Annual Interim Report, Updated Protocol and SAP for Pfizer Study C4951006 Additional Survey Study Report for C4951006

17.3. Results of PASS imposed in the marketing authorisation(s)²⁹

None

17.4. Results of PASS non-imposed in the marketing authorisation(s)³⁰

17.4.1. Eslicarbazepine acetate – ZEBINIX (CAP) – EMA/VR/0000332409

Applicant: Bial Portela & Ca S.A.

PRAC Rapporteur: Dennis Lex

Scope: Submission of the final report from the post authorisations safety study EURAP (BIA-2093-402) listed as a category 3 study in the RMP. This is an international, prospective observational registry designed to assess the risks associated with antiepileptic drug exposure during pregnancy. The updated RMP version 23.0 has also been submitted. Risk information has been updated based on clinical evidence, including clinical trials and post-marketing data, together with a comprehensive review of the published literature.

17.4.2. Nirmatrelvir / Ritonavir – PAXLOVID (CAP) – EMA/VR/0000339731

Applicant: Pfizer Europe MA EEIG

PRAC Rapporteur: Dennis Lex

Scope: Submission of the final report from study C4671037, listed as a category 3 study in the RMP. This is a non-interventional post-authorisation safety study to assess the safety of Paxlovid in pregnant women.

17.4.3. Radium-223 – XOFIGO (CAP) – EMA/VR/0000333243

Applicant: Bayer AG

PRAC Rapporteur: Rugile Pilviniene

Scope: Submission of final study report from study 20511 listed in Annex IID of the SmPC. This is a phase I biodistribution study to further characterise correlation between the extent of the disease, the dose and the distribution of radium-223 in bone metastases versus sites of impaired bone health versus normal bone structure. The Annex II is updated accordingly.

17.5. Interim results and other post-authorisation measures for imposed and non-imposed studies

17.5.1. Diroximel fumarate – VUMERITY (CAP) – EMA/PAM/0000343706

Applicant: Biogen Netherlands B.V.

PRAC Rapporteur: Dennis Lex

²⁹ In accordance with Article 107p-q of Directive 2001/83/EC

³⁰ In accordance with Article 61a (6) of Regulation (EC) No 726/2004, in line with the revised variations regulation for any submission as of 4 August 2013

Scope: Third interim report for registry-based PASS 272MS403 (An Observational Study Utilising Data From Big MS Data Registries to Evaluate the Long-Term Safety of Vumerity and Tecfidera) - Cat 3.

17.5.2. Pegvaliase – PALYNZIQ (CAP) – EMA/PAM/0000319171

Applicant: Biomarin International Limited

PRAC Rapporteur: Rhea Fitzgerald

Scope: Third interim study report for study BM 165-501: a multi-centre observational study to evaluate the long-term safety of subcutaneous injections of Palynziq (pegvaliase) in subjects with phenylketonuria.

17.5.3. Pegvaliase – PALYNZIQ (CAP) – EMA/PAM/0000317250

Applicant: Biomarin International Limited

PRAC Rapporteur: Rhea Fitzgerald

Scope: Third interim report for study BM 165-504: a global multicentre study to assess maternal, foetal and infant outcomes of exposure to Palynziq (pegvaliase) during pregnancy and breastfeeding.

17.5.4. Respiratory syncytial virus mRNA vaccine (nucleoside modified) – MRESVIA (CAP) – EMA/PAM/0000316615

Applicant: Moderna Biotech Spain S.L.

PRAC Rapporteur: Jean-Michel Dogné

Scope: Assessment of Post-Authorisation Active Surveillance Safety Studies Using Secondary Data to Monitor Real-World Safety of the mRNA-1345 Vaccine for respiratory syncytial virus (RSV) in the United States and in Europe (interim results for mRNA-1345-P902 and mRNA-1345-P903).

17.5.5. Selexipag – UPTRAVI (CAP) – EMA/PAM/0000309454

Applicant: Janssen Cilag International

PRAC Rapporteur: Zoubida Amimour

Scope: Second interim Clinical study report of study AC-065A403 (EDUCATE), a category 3 PASS study (EMA/H/C/003774/MEA/003) with a data cut-off date of 11 July 2025.

17.5.6. Tezepelumab – TEZSPIRE (CAP) – EMA/PAM/0000343681

Applicant: AstraZeneca AB

PRAC Rapporteur: Eva Jirsová

Scope: Submission of the interim progress report for study D5180R00024, a non-interventional, multi-country Post-Authorisation Safety Study (PASS) evaluating the incidence of serious cardiac events in patients with severe uncontrolled asthma treated with tezepelumab. This PASS is listed as category 3 in the RMP.

17.6. New Scientific Advice

Disclosure of information related to this section cannot be released at the present time as it is deemed to contain commercially confidential information.

17.7. Ongoing Scientific Advice

Information related to this section cannot be released at the present time as it is deemed to contain commercially confidential information.

17.8. Final Scientific Advice (Reports and Scientific Advice letters)

Information related to this section cannot be released at the present time as it is deemed to contain commercially confidential information.

18. Annex I – Renewals of the marketing authorisation, conditional renewals and annual reassessments

Based on the review of the available pharmacovigilance data for the medicine(s) listed below and the CHMP Rapporteur's assessment report, PRAC considered that either the renewal of the marketing authorisation procedure could be concluded - and supported the renewal of their marketing authorisations for an unlimited or additional period, as applicable - or no amendments to the specific obligations of the marketing authorisation under exceptional circumstances for the medicines listed below were recommended. As per the agreed criteria, the procedures were finalised at the PRAC level without further plenary discussion.

18.1. Annual reassessments of the marketing authorisation

18.1.1. Chenodeoxycholic acid – CHENODEOXYCHOLIC ACID LEADIANT (CAP) – EMA/S/0000340415

Applicant: Leadiant GmbH

PRAC Rapporteur: Adam Przybylkowski

Scope: Annual reassessment of the marketing authorisation

18.1.2. Idursulfase – ELAPRASE (CAP) – EMA/S/0000339599

Applicant: Takeda Pharmaceuticals International AG Ireland Branch

PRAC Rapporteur: Liana Martirosyan

Scope: Annual reassessment of the marketing authorisation

18.1.3. Susoctocog alfa – OBIZUR (CAP) – EMA/S/0000324538

Applicant: BAXALTA INNOVATIONS GmbH

PRAC Rapporteur: Dirk Mentzer

Scope: Annual reassessment of the marketing authorisation

18.1.4. Tabelecleucel – EBVALLO (CAP) – EMA/S/0000326533

Applicant: Pierre Fabre Medicament

PRAC Rapporteur: Amelia Cupelli

Scope: Annual reassessment of the marketing authorisation

18.1.5. Zanamivir – DECTOVA (CAP) – EMA/S/0000339464

Applicant: Glaxosmithkline Trading Services Limited

PRAC Rapporteur: Jenny-Maria Jönsson

Scope: Annual reassessment of the marketing authorisation

18.2. Conditional renewals of the marketing authorisation

18.2.1. Elranatamab – ELREXFIO (CAP) – EMA/R/0000347279

Applicant: Pfizer Europe MA EEIG

PRAC Rapporteur: Barbara Kovacic Bytyqi

Scope: Conditional renewal of the marketing authorisation

18.3. Renewals of the marketing authorisation

18.3.1. Avacopan – TAVNEOS (CAP) – EMA/R/0000340631

Applicant: Vifor Fresenius Medical Care Renal Pharma France

PRAC Rapporteur: Liana Martirosyan

Scope: 5-year renewal of the marketing authorisation

18.3.2. Finerenone – KERENDIA (CAP) – EMA/R/0000340433

Applicant: Bayer AG

PRAC Rapporteur: Bianca Mulder

Scope: 5-year renewal of the marketing authorisation

18.3.3. Formoterol / Glycopyrronium bromide / Budesonide – RILTRAVA AEROSPHERE (CAP) – EMA/R/0000340519

Applicant: AstraZeneca AB

PRAC Rapporteur: Jan Neuhauser

Scope: 5-year renewal of the marketing authorisation

18.3.4. Lonapegsomatropin – SKYTROFA (CAP) – EMA/R/0000339532

Applicant: Ascendis Pharma Endocrinology Division A/S

PRAC Rapporteur: Dennis Lex

Scope: 5-year renewal of the marketing authorisation

18.3.5. [Pneumococcal polysaccharide conjugate vaccine \(20-valent, adsorbed\) – PREVENAR 20 \(CAP\) – EMA/R/0000340648](#)

Applicant: Pfizer Europe MA EEIG

PRAC Rapporteur: Jean-Michel Dogné

Scope: 5-year renewal of the marketing authorisation

18.3.6. [Rivaroxaban – RIVAROXABAN VIATRIS \(CAP\) – EMA/R/0000327079](#)

Applicant: Viatris Limited

PRAC Rapporteur: Karin Bolin

Scope: 5-year renewal of the marketing authorisation

18.3.7. [Semaglutide – WEGOVY \(CAP\) – EMA/R/0000336288](#)

Applicant: Novo Nordisk A/S

PRAC Rapporteur: Karin Bolin

Scope: 5-year renewal of the marketing authorisation

18.3.8. [Somatrogen – NGENLA \(CAP\) – EMA/R/0000340629](#)

Applicant: Pfizer Europe MA EEIG

PRAC Rapporteur: Liana Martirosyan

Scope: 5-year renewal of the marketing authorisation

18.3.9. [Tecovirimat – TECOVIRIMAT SIGA \(CAP\) – EMA/R/0000340615](#)

Applicant: Siga Technologies Netherlands B.V.

PRAC Rapporteur: Dennis Lex

Scope: 5-year renewal of the marketing authorisation

18.3.10. [Tepotinib – TEPMETKO \(CAP\) – EMA/R/0000339701](#)

Applicant: Merck Europe B.V.

PRAC Rapporteur: Bianca Mulder

Scope: 5-year renewal of the marketing authorisation

19. Annex II – List of participants

including any restrictions with respect to involvement of members/alternates/experts following evaluation of declared interests for the 06-09 July 2026 PRAC meeting, which was held in-person.

An asterisk (*) after the role, in the second column, signals that the member/alternate attended remotely. Additional experts participated in (part of) the meeting, either in person or remotely.

Name	Role	Member state or affiliation	Outcome restriction following evaluation of DoI	Topics for which restriction apply
Ulla Wändel Liminga	Chair	Sweden	No interests declared	
Jan Neuhauser	Member*	Austria	No interests declared	
Sonja Radowan	Alternate*	Austria	No interests declared	
Jo Robays	Alternate	Belgium	No interests declared	
Maria Popova-Kiradjieva	Member	Bulgaria	No interests declared	
Petar Mas	Member	Croatia	No interests declared	
Barbara Kovacic Bytyqi	Alternate	Croatia	No interests declared	
Elena Kaisis	Alternate	Cyprus	No interests declared	
Veronika Macurova	Alternate	Czechia	No interests declared	
Marie Louise Schougaard Christiansen	Member	Denmark	No interests declared	
Karin Erneholm	Alternate	Denmark	No interests declared	
Maia Uusküla	Member*	Estonia	No interests declared	
Krõõt Aab	Alternate*	Estonia	No interests declared	
Terhi Lehtinen	Member	Finland	No interests declared	
Kimmo Jaakkola	Alternate*	Finland	No interests declared	
Tiphaine Vaillant	Member	France	No interests declared	

Name	Role	Member state or affiliation	Outcome restriction following evaluation of DoI	Topics for which restriction apply
Zoubida Amimour	Alternate	France	No participation in discussion, final deliberations and voting on:	15.3.15. EMA/VR/00003 24950 15.3.19. EMA/VR/00003 43906 16.1.2. EMA/PSUR/000 0336084
Dennis Lex	Member	Germany	No interests declared	
Dirk Mentzer	Alternate	Germany	No interests declared	
Georgia Gkegka	Member	Greece	No interests declared	
Maria Poulianiti	Alternate*	Greece	No participation in discussion, final deliberations and voting on:	16.3.19. EMA/PSUR/000 0336110
Julia Pallos	Member	Hungary	No participation in discussion, final deliberations and voting on:	15.3.15. EMA/VR/00003 24950 15.3.19. EMA/VR/00003 43906 16.1.2. EMA/PSUR/000 0336084
Rhea Fitzgerald	Member	Ireland	No interests declared	
Eamon O Murchu	Alternate	Ireland	No interests declared	
Amelia Cupelli	Member	Italy	No interests declared	
Zane Neikena	Member	Latvia	No interests declared	
Diana Litenboka	Alternate*	Latvia	No interests declared	
Rugile Pilvinienė	Member	Lithuania	No restrictions	

Name	Role	Member state or affiliation	Outcome restriction following evaluation of DoI	Topics for which restriction apply
			applicable to this meeting	
Anne-Cecile Vuillemin	Member	Luxembourg	No interests declared	
John Joseph Borg	Member*	Malta	No restrictions applicable to this meeting	
Liana Martirosyan	Member	Netherlands	No interests declared	
Bianca Mulder	Alternate	Netherlands	No restrictions applicable to this meeting	
David Olsen	Member	Norway	No participation in discussion, final deliberations and voting on:	4.2.2. Levonorgestrel intrauterine device 13.5 mg (Jaydess/Fleree) (NAP) 6.1.1. EMA/PSUR/000 0336116 16.3.9. EMA/PSUR/000 0336094 16.3.13. EMA/PSUR/000 0336102 17.4.3. EMA/VR/00003 33243 18.3.2. EMA/R/000034 0433
Pernille Harg	Alternate	Norway	No interests declared	
Adam Przybylkowski	Member	Poland	No restrictions applicable to this meeting	
Ana Sofia Diniz Martins	Member	Portugal	No interests declared	

Name	Role	Member state or affiliation	Outcome restriction following evaluation of DoI	Topics for which restriction apply
Roxana Dondera	Member*	Romania	No interests declared	
Roxana Stefania Udrescu	Alternate	Romania	No interests declared	
Miroslava Gocova	Member	Slovakia	No interests declared	
Jana Pecherova	Alternate	Slovakia	No interests declared	
Marjetka Plementas	Alternate	Slovenia	No interests declared	
Maria del Pilar Rayon	Member	Spain	No interests declared	
Maria Martinez Gonzalez	Alternate	Spain	No interests declared	
Karin Bolin	Member	Sweden	No restrictions applicable to this meeting	
Jenny-Maria Jönsson	Alternate	Sweden	No restrictions applicable to this meeting	
Annalisa Capuano	Member*	Independent scientific expert	No restrictions applicable to this meeting	
Maria Teresa Herdeiro	Member	Independent scientific expert	No restrictions applicable to this meeting	
Patricia McGettigan	Member	Independent scientific expert	No restrictions applicable to this meeting	
Hedvig Marie Egeland Nordeng	Member	Independent scientific expert	No restrictions applicable to this meeting	
Anette Kirstine Stark	Member	Independent scientific expert	No restrictions applicable to this meeting	

Name	Role	Member state or affiliation	Outcome restriction following evaluation of DoI	Topics for which restriction apply
Roberto Frontini	Member	Healthcare Professionals' Representative	No restrictions applicable to this meeting	
Martin Votava	Alternate	Healthcare Professionals' Representative	No restrictions applicable to this meeting	
Yiannoula Koulla	Member	Patients' Organisation Representative	No interests declared	
Michal Rataj	Alternate	Patients' Organisation Representative	No interests declared	
Martine Sabbe	Expert	Belgium	No interests declared	
Chloé Wyndham-Thomas	Expert	Belgium	No restrictions applicable to this meeting	
Selma Arapovic Dzakula	Expert	Croatia	No interests declared	
Marina Lesicar	Expert	Croatia	No interests declared	
Jana Kopecka	Expert	Czech Republic	No interests declared	
Lucie Skálová	Expert	Czech Republic	No interests declared	
Nicklas Hasselblad Lundstrøm	Expert	Denmark	No interests declared	
Sofie Lindholm	Expert	Denmark	No interests declared	
Cecilie Louise Pedersen	Expert	Denmark	No participation in discussion, final deliberations and voting on:	4.1.1. Atogepant - AQUIPTA (CAP); Erenumab - AIMOVIG (CAP); eptinezumab - VYEPTI (CAP); fremanezumab - AJOVY (CAP); galcanezumab - EMGALITY (CAP);

Name	Role	Member state or affiliation	Outcome restriction following evaluation of DoI	Topics for which restriction apply
				rimegepant - VYDURA (CAP) 5.1.1. EMEA/H/C/006344 15.3.17. EMA/VR/0000322527 15.3.26. EMA/VR/0000327359 18.3.7. EMA/R/0000336288
Pauline Dayani	Expert	France	No interests declared	
Sandrine El Khoury	Expert	France	No interests declared	
Birte Niemann	Expert	Germany	No interests declared	
Gareth Bowen	Expert	Ireland	No interests declared	
Sheena Kennedy	Expert	Ireland	No restrictions applicable to this meeting	
Lina Masioniene	Expert	Lithuania	No interests declared	
Gerlienke Geurts-Voerman	Expert	Netherlands	No interests declared	
Marianne Klanker	Expert	Netherlands	No interests declared	
Peter Mol	Expert	Netherlands	No restrictions applicable to this meeting	
Hester Peltenburg	Expert	Netherlands	No interests declared	
Siona Slob	Expert	Netherlands	No interests declared	
Dennis van Eijl	Expert	Netherlands	No interests declared	

Name	Role	Member state or affiliation	Outcome restriction following evaluation of DoI	Topics for which restriction apply
Inge Zomerdijs	Expert	Netherlands	No interests declared	
Iwona Korzeniewska-Rybicka	Expert	Poland	No restrictions applicable to this meeting	
Consuelo Argumánez	Expert	Spain	No interests declared	
Faiez Al Nimer	Expert	Sweden	No restrictions applicable to this meeting	
Charlotte Backman	Expert	Sweden	No interests declared	
Rolf Gedeberg	Expert	Sweden	No restrictions applicable to this meeting	
Miriam Taekema	Expert	Sweden	No interests declared	
A representative from the European Commission attended the meeting.				
Observers from Health Canada and SwissMedic attended the meeting.				
Meeting run with support from relevant EMA staff				
Experts were evaluated against the agenda topics or activities they participated in.				

20. Annex III - List of acronyms and abbreviations

For a list of acronyms and abbreviations used in the PRAC minutes, see:

[List of abbreviations used in EMA human medicines scientific committees and CMDh documents, and in relation to EMA's regulatory activities](#)

21. Explanatory notes

The Notes give a brief explanation of relevant minute's items and should be read in conjunction with the minutes.

EU Referral procedures for safety reasons: Urgent EU procedures and Other EU referral procedures

(Items 2 and 3 of the PRAC minutes)

A referral is a procedure used to resolve issues such as concerns over the safety or benefit-risk balance of a medicine or a class of medicines. In a referral, EMA is requested to conduct a scientific assessment of a particular medicine or class of medicines on behalf of the European Union (EU). For further detailed

information on safety related referrals please see: [Referral procedures: human medicines | European Medicines Agency \(europa.eu\)](https://www.ema.europa.eu/en/referral-procedures-human-medicines)

Signals assessment and prioritisation

(Item 4 of the PRAC minutes)

A safety signal is information on a new or incompletely documented adverse event that is potentially caused by a medicine and that warrants further investigation. Signals are generated from several sources such as spontaneous reports, clinical studies and the scientific literature. The evaluation of safety signals is a routine part of pharmacovigilance and is essential to ensuring that regulatory authorities have a comprehensive knowledge of a medicine's benefits and risks.

The presence of a safety signal does not mean that a medicine has caused the reported adverse event. The adverse event could be a symptom of another illness or caused by another medicine taken by the patient. The evaluation of safety signals is required to establish whether or not there is a causal relationship between the medicine and the reported adverse event.

The evaluation of safety signals may not necessarily conclude that the medicine caused the adverse event in question. In cases where a causal relationship is confirmed or considered likely, regulatory action may be necessary and this usually takes the form of an update of the summary of product characteristics and the package leaflet.

Risk Management Plans (RMPs)

(Item 5 of the PRAC minutes)

The RMP describes what is known and not known about the side effects of a medicine and states how these risks will be prevented or minimised in patients. It also includes plans for studies and other activities to gain more knowledge about the safety of the medicine and risk factors for developing side effects. RMPs are continually modified and updated throughout the lifetime of the medicine as new information becomes available.

Assessment of Periodic Safety Update Reports (PSURs)

(Item 6 of the PRAC minutes)

A PSUR is a report providing an evaluation of the benefit-risk balance of a medicine, which is submitted by marketing authorisation holders at defined time points following a medicine's authorisation. PSURs summarises data on the benefits and risks of a medicine and includes the results of all studies carried out with this medicine (in the authorised and unauthorised indications).

Post-authorisation Safety Studies (PASS)

(Item 7 of the PRAC minutes)

A PASS is a study of an authorised medicinal product carried out to obtain further information on its safety, or to measure the effectiveness of risk management measures. The results of a PASS help regulatory agencies to evaluate the safety and benefit-risk profile of a medicine.

Product related pharmacovigilance inspections

(Item 9 of the PRAC minutes)

Inspections carried out by regulatory agencies to ensure that marketing authorisation holders comply with their pharmacovigilance obligations.

Article 58 of Regulation (EC) No 726/2004 (EU-M4all)

Article 58 (EU-M4all) procedure allows the Committee for Medicinal Products for Human Use (CHMP) to give opinions, in co-operation with the World Health Organisation (WHO) on medicinal products for human use that are intended exclusively for markets outside of the European Union (EU).

More detailed information on the above terms can be found on the EMA website:
<https://www.ema.europa.eu/en>